

GENERAL HOSPITAL OPERATIONAL POLICY

**Hospital Pengajar
Universiti Sultan Zainal Abidin**



This policy was produced by
Quality & Standards Unit,
Hospital Pengajar Universiti Sultan Zainal Abidin 2023

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HOSPITAL PENGAJAR UNIVERSITI SULTAN ZAINAL ABIDIN



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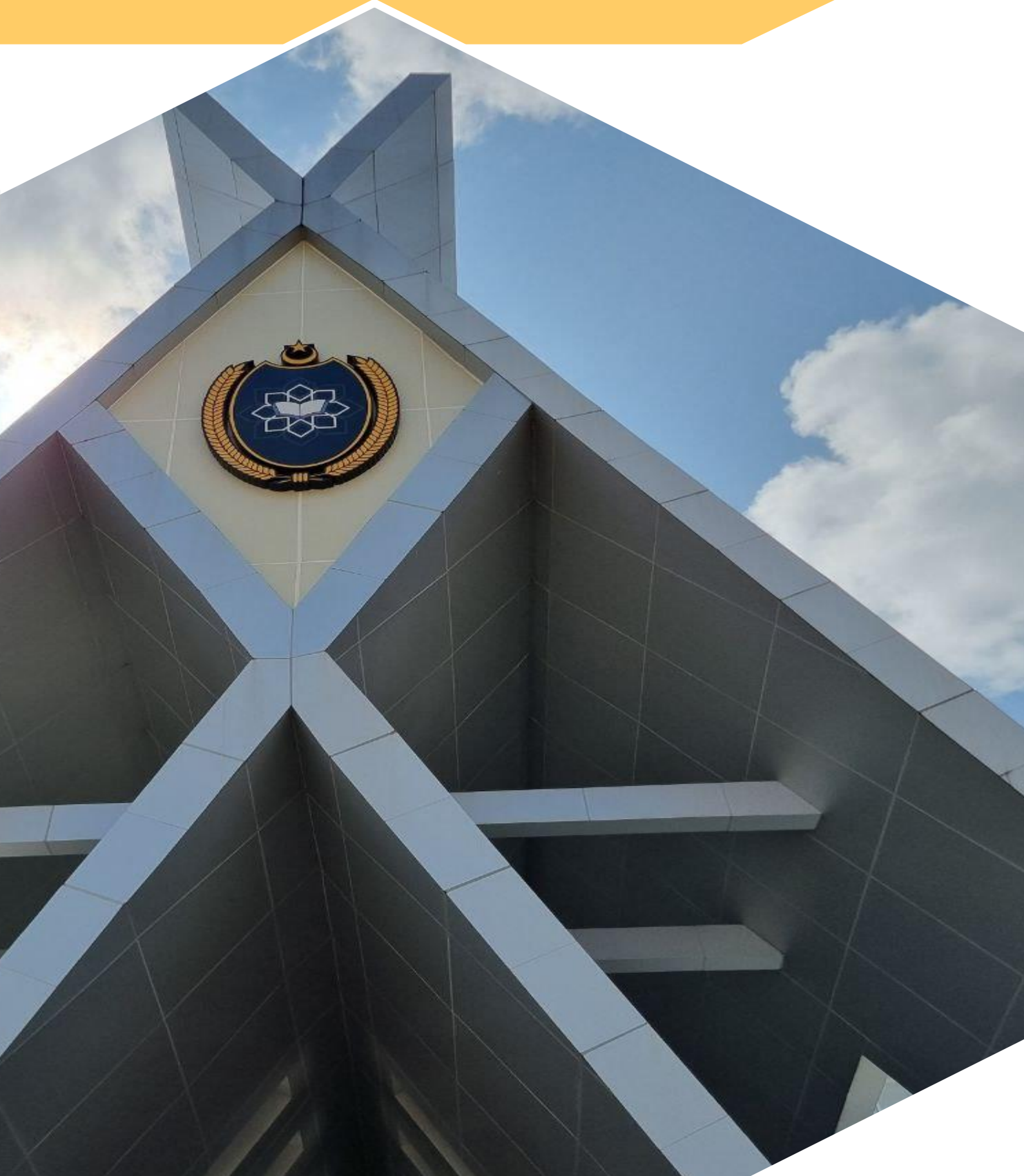
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FOREWORD & PREFACE



FOREWORD

We are extremely honoured and privileged to witness the completion of the General Hospital Operational Policy (GHOP) which was made possible with the hard work and involvement of various parties including clinicians, support services and corporate governance.

This policy was created following our hospital staffs' frequent encounter with different issues for which information already exists, either in documented form or undocumented practices. This information may be fragmented, scattered and unevaluated. Realizing this, efforts were undertaken to collect such information, analyse and then document them in an accessible and friendly form.

GHOP was primarily established as a documentation of guidelines to assist our staff members in their efforts to give excellent service to the Hospital and University. This protocol provides clarity to hospital policies and practices, thus helping staff members to make better decisions in providing outstanding healthcare services to the Ummah. GHOP covers five sections, of which the most important sections, I and II, describe in full the hospital's policies and basic principles. All the phases are covered; from the moment of patients' arrival to the hospital to their discharge, hospital organisational charts to disaster management and contingency plan.

We hope that all our staff members make full use of this guidebook, to understand the fundamentals of the management of the hospital and to excel in its daily operation.

We are grateful for the timely efforts brought forth by the editorial panel and the secretariat of GHOP. We truly believe that GHOP will be a helpful guide and source of information for numerous practical applications. Jazakumullahu khairan to everyone who have made this monumental journey possible.

"Knowledge for the Benefit of Humanity"
(*Ilmu Demi Faedah Insan*)



Dato' Profesor Dr. Fadzli bin Adam
Vice Chancellor,
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PREFACE

Alhamdulillah, all praise is for Allah, Lord of all the worlds, the utmost Kind, the ever Merciful. We are very happy to introduce our very own General Hospital Operating Policy (GHOP), which illustrates the Hospital and the University's capability and commitment to serve the Ummah with the best of our capacity.

The initiative taken to formulate the GHOP for Hospital Pengajar Universiti Sultan Zainal Abidin (HPUniSZA) serves as a road-map for the day-to-day operations of HPUniSZA. GHOP provide us a proper guidance to standardize our framework to facilitate the hospital staff in delivering excellent healthcare, bringing us to the next level in patient safety and well-being. In the end, this important policy will result in a safer and more conducive workplace to the hospital staff and patients alike. However, this policy needs to be reviewed from time to time, to be adjusted, adapted to the new technology and definitely to accommodate the changes that may take place for the benefit of hospital personnel, patients and the country.

This document contains general policies related to the management and operations of the hospital. It also contains the policies on hospital and clinical governance, disaster management and contingency plans for the hospital.

We sincerely hope that all of the Head of Departments in the hospital are familiar with the operational policy and hence shall ensure all departmental staff are aware and well-informed about these policies and ready to deliver. It is our aspiration that this policy will be the leading principles in bringing the Hospital and the University to greater heights together.



Prof. Dato' Dr. Ahmad Zubaidi bin A.Latif
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The background features a large yellow triangle pointing downwards, which is partially overlaid by a black triangle pointing upwards. The number '2' is positioned to the right of the black triangle.

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THE POLICY

Chapter 1:

Organisational Overview

University Sultan Zainal Abidin Teaching Hospital (HPUniSZA) is committed to provide consistent and excellent healthcare. The hospital revolves around the principles outlined below that are consistent with the university's aspiration. Our organisation is structured to provide robust governance to provide clarity for staff members.

1.1 Vision, Mission, Motto and Objectives

1.1.1 Vision:

- To be the Leading Teaching Hospital and tertiary referral centre for the country.

1.1.2 Mission:

- To provide comprehensive, excellent and holistic health care services
- To provide high quality teaching and training for students and staffs
- To cooperate with the public and private medical facilities.
- To achieve financial sustainability.

1.1.3 Motto

- Health, the heart of Life (***"Kesihatan Nadi Kehidupan"***)

1.1.4 Objectives:

- To establish high quality healthcare service and incorporating Islamic values wherever possible.
- To become a world class training centre for undergraduates and postgraduate students.
- To collaborate with public and private hospitals, universities and related institutions.
- To establish itself as the referral centre for the East Coast initially then eventually to the country.
- To be a world-class research and innovation centre.
- To produce high quality publications in health and medicine.
- To establish financial management sustainability through income generation from the various services.
- To carry out regular health activities and promotions at the state, national and international levels.
- To support the regional health tourism industry.

1.2 Speciality Fields

We are focused on delivering health care services through:

- 1.2.1 Cardiology
- 1.2.2 Hyperbaric Oxygen Therapy (HBOT)
- 1.2.3 Urology
- 1.2.4 Interventional Radiology
- 1.2.5 Ambulatory & Daycare Community

1.3 Core Values

Our core values uphold an enduring set of values which are ***Ihsan*, Professionalism, Respect, Integrity** and **Trustworthiness**.

- 1.3.1 ***Ihsan***
- 1.3.2 **Professionalism**
- 1.3.3 **Respect**
- 1.3.4 **Integrity**
- 1.3.5 **Trustworthiness**

1.4 Principal Strategies

- 1.4.1 World Class Health Service Delivery.
- 1.4.2 High Impact Education.
- 1.4.3 Empowering Teaching and Learning.
- 1.4.4 Community Heroes
- 1.4.5 Financial Sustainability.
- 1.4.6 Shariah-Compliant Governance.

1.5 Organisational Structure

- 1.5.1 Hospital Overview
 - i. HPUniSZA is located at the UniSZA Gong Badak Campus, Kuala Nerus, Terengganu.
 - ii. Primarily an academic healthcare facility, HPUniSZA strives to be a tertiary referral centre for the region.
 - iii. HPUniSZA is a specialist-led hospital with a total of 400 beds capacity, providing comprehensive Clinical Services which include a wide range of Medical, Surgical and Diagnostic Services. The Medical Services consist of

seven (7) departments and four (4) units while the Surgery Services consists of six (6) departments and Diagnostic Service consists of five (5) departments. HPUniSZA also has Support Services from Clinical and Non-clinical services. The Clinical Support Services consist of five (5) departments and two (2) units while the Non-Clinical Support Services consist of three (3) departments and one (1) unit.

- iv. This hospital will complement the existing services provided by the Ministry of Health (MOH) as well the General Practitioners and Private Hospitals or Medical Centres.

1.5.2 Organisational Governance

- i. The hospital is led by the Hospital Director who will be appointed by the Chairman of the University Board of Directors. The Hospital Director shall be an employee of HPUniSZA and is responsible for the overall management of the hospital and staff.
- ii. There shall be a Deputy Director who is in charge of Clinical Services and a Deputy Director who is in charge of Non-Clinical Support Services.
- iii. In the absence of the Hospital Director, the Deputy Director shall perform his/her duties and assume the responsibilities of the Hospital Director.
- iv. There shall be a Deputy Director of Administration and Management to oversee the administrative, management and security matters of the Hospital.
- v. Each Clinical Department shall be headed by a Head of Department or, in his/her absence, by the most senior consultant appointed by the Hospital Director.
- vi. Non-Clinical Departments shall be headed by officers trained in their respective disciplines.
- vii. The duties and responsibilities of the Head of Departments shall be directed by the Hospital Director following the functions, aspirations and objectives of the Hospital.
- viii. There shall be a Head of Nursing Department to oversee the nursing services of the Hospital. The Chief of Nursing Officer (*Ketua Jabatan Kejururawatan*) may be assisted by a Nurse Officer (*Pegawai Jururawat*), Nurse Supervisor (*Penyelia Jururawat*) and Nurse Manager (*Ketua Jururawat*).
- ix. The Nurse Officer, Nurse Supervisor and Nurse Manager will be responsible for the Central Sterile Service Department (CSSD), laundry and linen services, and infection control.
- x. There shall be a Head of Hospital Supervision Unit who shall be responsible for coordinating services provided by the Assistant Medical Officer (AMO), Health Care Assistant (PPK), Porterage and ambulance. AMO shall manage for ambulance service of the Hospital under

supervision of Emergency and Trauma Department (ETD).

- xi. The Department of Engineering and Biomedical shall be headed by the Head of Engineering, assisted by engineers, assistant engineers and technicians. The engineer shall ensure aspects related to outsourced services including hospital waste management, fire safety, maintenance of grounds, landscaping, maintenance and repair of all civil, mechanical, electrical and biomedical installations are appropriately undertaken by the appointed company.
- xii. The Department of Information Technology shall be headed by the Head of Information Technology. He/she shall be responsible for the planning operations, maintenance and monitoring of Information & Communication Technology (ICT) systems installed in the Hospital.

1.5.3 Organisational Structure

A. Organisational Chart

Refer to Appendix 1: HPUniSZA Organisational Chart

B. Committee

- i. The establishment of committees and working committees under the Hospital shall be subject to the laws and regulations of HPUniSZA.
- ii. To ensure good practice, the Hospital may refer to the circular from the Director of Health (DG) Ministry of Health, Malaysia to facilitate the operations of the Hospital.^{1[4]}
- iii. There shall be committees in HPUniSZA that overlook the clinical support services and the administration services to ensure the smooth delivery of care and running of the Hospital.

1.5.4 Directorate

HPUniSZA shall establish directorates which include but not limited to the following:

A. Clinical Directorate

- i. Department of Internal Medicine
- ii. Department of Paediatric
- iii. Department of Emergency and Trauma
- iv. Department of Psychiatry and Mental Health
- v. Department of Rehabilitation Medicine
- vi. Department of Family Medicine
- vii. Infection Control Unit

^[4] Surat Pekeliling Ketua Pengarah Kesihatan Bil 7/2000 – Garis Panduan Struktur Organisasi Pengurusan Hospital. Reference No: (15) in KKM-87(P20/702), 22 Disember 2000.

- viii. Pre-Hospital Care Unit
 - ix. Department of Community Medicine
 - x. Medical Education and Health Promotion Unit
 - xi. Occupational Health and Public Health Unit
- B. Surgical Directorate
 - i. Department of Surgery
 - ii. Department of Obstetrics and Gynaecology
 - iii. Department of Anaesthesiology & Intensive Care
 - iv. Department of Orthopaedics
 - v. Department of Ophthalmology
 - vi. Department of Otorhinolaryngology
- C. Diagnostic Directorate
 - i. Department of Medical Imaging
 - ii. Department of Pathology & Medical Laboratory
 - iii. Department of Forensic Medicine
 - iv. Department of Anatomy & Physiology
 - v. Department of Pharmacology
- D. Clinical Support Directorate
 - i. Department of Pharmacy
 - ii. Department of Nursing
 - iii. Department of Dietetics and Food Serving
 - iv. Department of Central Sterile Supply (CSSD)
 - v. Department of Medical Education and Health Professions
 - vi. Hospital Supervision Unit
 - vii. Medical Social Work Unit
- E. Non-Clinical Support Directorate
 - i. Department of Engineering & Biomedical
 - ii. Department of Medical Record
 - iii. Department of Information Technology
 - iv. Quality & Standards Unit

D. Management Directorate

- i. Department of Human Resource
- ii. Department of Finance
- iii. Department of Business Development and Investment
- iv. Department of Governance and Administration
- v. Safety Unit
- vi. Library Unit

E. Director Office

- i. Legal Unit
- ii. Corporate Communication & Customer Services Unit (CCaCS)
- iii. Religious Affairs Unit
- iv. Clinical Research Unit

1.6 Patients Charter

HPUniSZA is committed to delivering ethical, quality and efficient services to each patient, abiding by the requirements of the available regulations and standards, at the same time protecting the rights of the patient as below:

- i. Patients will be provided with professional and customer-friendly services.
- ii. Patients may have access to medical information which relates to their condition and treatment. Medical Reports will be provided on request.
- iii. Patients may accept or refuse any medication, investigation or treatment, and those who refuse, will be counselled on the likely consequences or repercussions, as well as provided with alternatives where available. Informed consent will be taken for any invasive procedure and/or surgery.
- iv. All patients will be ensured confidentiality of their medical conditions; however, this may be revealed to third parties, such as the Insurers upon request.
- v. Grievances will be dealt with through a special channel head by a dedicated team.
- vi. As part of the requirement of a teaching hospital, patients shall agree to be examined by medical students for teaching and examination purposes. An opt-out option will only be allowed in selected instances.
- vii. Patients may be subjected to research, in which case adherence to the latest Good Clinical Practice (GCP) guidelines will be implemented.^[1]

^[1] Malaysian Guideline for Good Clinical Practice 4th Edition 2018

Chapter 2:

Management and Operations

2.1 General Administration

2.1.1 Letters and Documents

- i. The Department of Governance and Administration shall be responsible for the management of all incoming and outgoing official letters and e-mail communication.
- ii. The Hospital shall have a common and systematic Hospital filing system of official documents. Both incoming and outgoing letters shall be filed accordingly.
- iii. Incoming letters/documents, whether in electronic or hard copy form, shall be registered, minutes recorded and dispatched to the respective department/unit within a specified time. Urgent letters shall be dispatched immediately, and the respective department/unit informed via telephone communication.
- iv. All outgoing official letters shall use the standard letterhead of the Hospital.
- v. Letters for internal circulation shall be circulated as Memos.
- vi. Confidential letters and documents shall be handled according to the relevant laws and regulations.
- vii. Letters and documents shall be kept for the required number of years. Disposal of letters and documents shall be in accordance with the procedures and guidelines issued by HPUniSZA and the Department of National Archive.^{2[5]}
- viii. All other departments in the Hospital shall adhere to this guideline when handling letters and documents.

2.1.2 Office Equipment and Supplies

- i. The Department of Governance and Administration shall coordinate the requirement of office equipment and supplies of the Hospital and distribution to the relevant units or departments.
- ii. The Head of Department of Governance and Administration shall be responsible for maintaining the asset and inventory list and ensuring proper and efficient use of equipment and supplies.
- iii. To ensure efficiency, certain office equipment may be shared among several departments or units. Shared equipment shall be under the responsibility of the department/unit where the equipment is/are located.
- iv. The Hospital assets shall be managed and administered according to the current practice directions and circulars of HPUniSZA.

[5] National archives Act 2003 (Act 629) Laws of Malaysia.

2.1.3 Meeting Room Facilities

- i. The use of meeting rooms and other facilities like auditoriums, etc. shall be coordinated according to the current practice directions and circulars of HPUniSZA.
- ii. The general meeting rooms shall be under the care of the Department of Governance and Administration.
- iii. The meeting room under a specific department/unit shall be under the care of the department/unit.

2.2 Finance

2.2.1 Budget

An adequate budget setting process shall be in place, in line with the Hospital's needs and objectives, according to the current practice directions of HPUniSZA.^[6]

A. Budget Preparation

- i. At any time not later than the fourth quarter of a particular year, the Hospital Director and Head of Department (Finance) shall convene a meeting with the Heads of Departments to provide guidelines for budget preparation.
- ii. The Heads of Departments shall be responsible for preparing estimates of the current year, as well as revenue and expenditure estimates for the upcoming year. The Heads of Departments must pay close attention to historical trends and current needs when determining estimates, programmed agreements and evaluation and prepare a financial report, if required, at the end of the budget year.
- iii. The Head of Department (Finance) shall prudently manage the budget, adhering to financial regulations and procedures and be responsible for formulating justifications for additional budget.
- iv. The *Jawatankuasa Bajet Universiti (JBU)* shall oversee finance and accounts related issues including expenditure status, budget reallocation and additional budget requirements.
- v. The Hospital Director shall be fully responsible for the management of the allocation and expenditure of the Hospital.

B. Budget Analysis

- i. Within ten days after the end of each month, a written budget analysis shall be performed by the *Mesyuarat Pengurusan Hospital Universiti (MPHU)* and by the quarterly report to the *Jawatankuasa Pengurusan Kewangan dan Akaun (JPKA)* consisting of the following elements:
 - Actual revenues and expenditures will be compared to budgeted

[6] Arahan Perbendaharaan 59 - Peraturan Kewangan Dan Perakaunan UniSZA.

amounts; both for the current month and the year-to-date.

- Significant variances will be noted and their causes determined.
- ii. The Hospital Director will forward the quarterly budget analysis, in whole or in summary, to the *Jawatankuasa Pengurusan Hospital Universiti (JPHU)* along with recommended actions, if necessary.

2.2.2 Cash Management

A. Cash Forecast

- i. Guidelines for preparing cash forecasts for HPUniSZA require a manual check.
- ii. Cash forecasts shall be prepared quarterly.
- iii. The Management shall be updated on the cash status at every management meeting.
- iv. All investments and borrowings for any amount shall require approval by the MPHU.

B. Cash Handling

All cash and bank transactions involving the handling of cash shall be carried out in accordance with the government procedures, practice directions and circulars of UniSZA and with proper authorisation, control, monitoring, and properly classified in the accounting records of the Hospital. The same level of care and diligence shall be carried out in the following transactions:

- i. Opening and authorisation of new bank accounts
- ii. Closing Bank Accounts
- iii. Check Issuance
- iv. Wire Transfers and Cash Disbursements
- v. Authorised Bank Signatories
- vi. Cash Receipts, Bank Reconciliations
- vii. Petty Cash

2.2.3 Procurement

All procurements including purchase authority limits shall be carried out in accordance with the government procedures, practice directions and circulars of UniSZA.^[7]

^[7] *Pekeliling Bendahari Bilangan 4 tahun 2012 - Garis panduan berkenaan kaedah perolehan bagi bekalan dan perkhidmatan secara pembelian terus.*

2.2.4 Payment

- i. The Finance Department shall be responsible for recording purchases and payments.
- ii. Accounts payable shall be reconciled periodically.
- iii. Payments made by or to the Hospital shall be processed, monitored and reconciled following the government procedures, practice directions and circulars of UniSZA.^[11]

2.2.5 Claims and Loans

- i. All claims and loans shall adhere to the government procedure, practice directions and circulars of UniSZA.^[11]
- ii. Staff are required to submit claim forms within the following month and in the same year except for December which can be claimed until the 10th of January. The claim forms have to be completed, signed and attached with the necessary documents.
- iii. Head of Department/Unit shall verify and validate the claims before submitting them to the Department of Finance.
- iv. Government and University loan applications shall be submitted based on eligibility and attached with the relevant forms and supporting documents.

2.2.6 Hospital Charges

- i. Fees shall be charged in accordance with the approval of MPHU.
- ii. Any new, additional or fee charge changes for procedures in the Fee Order shall be forwarded to the MPHU for approval.
- iii. The Hospital shall make available the information on Hospital fees or charges to all parties.
- iv. The deposit shall be collected prior to all admissions. The issuance of Letters of Guarantee for patients will be mandatory otherwise a minimum fee will be collected except in dire emergency situations.
- v. The Hospital shall take all possible measures to ensure proper patient payment collection. A Credit Control Department will be set up.
- vi. Any exemption or reduction to Hospital bills shall be subjected to approval by the *Jawatankuasa Pengurangan dan Pengecualian Caj (JKPP)*.

2.2.7 Billing and Payment

- i. For cash paying patients, the Hospital bill shall be given upon discharge and payment must be settled at the Admission and Revenue Counter (ARC) or the Department of Emergency and Trauma (ETD) revenue counter after office hours.

^[11] *Buku Panduan Program Medical Officer Siswazah, Edisi 2012, KKM.*

- ii. Out-patient bills shall be paid after any procedure within the same day either at the ARC counter or Out-patient Pharmacy.
- iii. Interim bills (summarized bills) shall be generated in the ward and given to the patient during their stay in the Hospital. Long staying patients shall be informed of their accumulated bills at intervals.
- iv. For patients with valid Letter of Guarantee, Hospital bills shall be sent to their employer. The Letter of Guarantee shall be submitted to the Revenue Counter (ARC) for verification and to be kept until the settlement of the Hospital bill.
- v. For appliances/materials not provided by the Hospital, patients shall be billed separately by the designated supplier and shall be paid accordingly by the patient (e.g. intraocular implant, orthopaedic implant etc.)
- vi. The Hospital shall receive payment in cash, money order, postal order, bank draft, bankers' cheque, online banking or credit/debit card. Personal cheques are not accepted.
- vii. Receipts (either paper or electronic) shall be issued upon payment.
- viii. Revenue collection shall be carried out by authorized personnel only at designated revenue counters.
- ix. A final bill shall be issued at the revenue counter or sent to the address stated in the Letter of Guarantee for settlement of the bill.

2.3 Human Resources

2.3.1 Human Resource Planning

- i. The Hospital management shall strive to provide appropriate numbers of people with the required skills in the Hospital. The Hospital management is responsible for human resource training in accordance with service needs and expansion plans.
- ii. Orientation
 - Newly appointed staff shall be informed about the terms and conditions of appointment in following the laws and regulations of UniSZA.^[3]
 - Newly appointed staff are required to attend orientation courses and/or programmes organised by UniSZA.
 - The orientation programme shall be organised for all new staff which includes an overall briefing on the Hospital policies, procedures, rules and regulation and their roles and responsibilities.
 - Specific briefings shall be given by the departments and units.
 - Hands-on training on ICT skills shall be arranged for all staff.

^[3] Statutory Bodies (Discipline And Surcharge) Act 2000 (Act 605)

iii. Placement

- Placement of staff to departments or units shall be based on qualification, specialised training received and service needs.
- The department/unit head shall be responsible for the placement and job description within the department/unit.
- Deployment and rotation of staff to other departments and units may be carried out as and when necessary.
- The Hospital Director is responsible to ensure the placement of sufficient ICT officers for the efficient running of the hospital.
- Reference may be made to the following documents regarding placement procedures in UniSZA.^{[2][10][11]}

iv. Work Attendance and Leave

- Staff shall record their daily attendance and movements within working hours using the appropriate person attendance system, subject to the current registrar circular. Staff requesting time-off during office hours shall complete the *Borang Kebenaran Untuk Meninggalkan Pejabat Dalam Waktu Bekerja*.
- Heads of Department/Unit shall be responsible for monitoring their staff's daily attendance/movement.
- Application for leave shall be done before taking the leave. The use of *Sistem E-Cuti* on the Staff Portal (Leave and time off application) for leave application is compulsory.
- Staff shall inform their Head of Department/Unit if he/she is not well to be present at work and/or has been given a Medical Certificate (MC).
- Staff participating in the Medical and Humanitarian Aid Mission shall be accorded a maximum of 14-days of non-recorded leave (including weekends and holidays) when the mission is organised by a recognised body.

2.3.2 Performance Evaluation

- Every staff shall have a MyPortfolio which contains the job description, responsibilities and related work guidelines and procedures.
- At the beginning of the year, the staff will discuss with his/her respective Head of Department to prepare his/her annual work target and Key Performance Indicators (KPI).

^[2] World Medical Association. (2013). Declaration of Helsinki, Ethical Principles for Scientific Requirements and Research Protocols. Bulletin of the World Health Organisation, 79(4), 373.- (<https://www.wma.net/policies-post/wma-declaration-of-helsinki-ethical-principles-for-medical-research-involving-human-subjects/>)

^[10] Approval of JKP (Locum), Ministry of Health Malaysia

^[11] *Buku Panduan Program Medical Officer Siswazah, Edisi 2012, KKM.*

- iii. The Head of Department/Unit shall monitor and evaluate his/her staff competencies.
- iv. The performance evaluation shall be carried out biannually and at appropriate intervals using the standard format in a just and fair manner.

2.3.3 Ethics and Discipline

- i. Behaviour at work and dress code:
 - The core values of HPUiSZA are ***Ihsan, Professionalism, Respect, Integrity and Trustworthiness*** shall be internalised and upheld by all staff while performing his/her duties.
 - During working hours, the staff shall professionally render services to uphold the image of the Hospital.
 - The Staff shall wear his/her respective uniform or proper working attire when he/she is at work.
 - Name tag and hospital identification card shall be worn at all times as part of the uniform/working attire. Ethics and discipline of all staff shall adhere to the current practice directions and circulars of UniSZA.^[13]
- ii. Disciplinary Problem
 - Monitoring of staff performance shall be continuous. Staff with disciplinary problems shall be given counselling before being referred for disciplinary action. Disciplinary actions shall be referred to the Integrity Unit and shall be subject to the laws and regulations of UniSZA.^[3]

2.3.4 Staff Welfare and Safety

- i. Hospital management shall provide a conducive environment for the staff to achieve organisational goals.
- ii. The Hospital shall establish *Jawatankuasa Keselamatan dan Kesihatan Pekerja* (JKKP) to look after the well-being and safety of the staff. (Refer to OSHA guidelines for details).
- iii. Clubs or associations may be created to provide opportunities for staff to get together, participate in sports or carry out other recreational activities.
- iv. The Hospital shall have an Ex-Gratia Work Disaster Scheme which in UniSZA is covered by the Social Security Organisation (SOCSO). This scheme provides compensation to Hospital staff stricken by disasters while carrying out work that causes permanent disability or death, including those victimised because of retaliation as a result of the action taken in the course of official duties. An officer is covered at all times while on official duty including:
 - While travelling to and from home to the workplace;
 - While travelling to and from the workplace to the residence at an

^[13] Pekeliling Pendaftar Bil. 15/2011 Etika Pakaian dan Sahsiah Rupa Diri bagi Staf HPUiSZA dan SPKPK Bil.1/2000 Amalan Etika Profesion Perubatan Yang Baik, 26 May 2000.

^[3] Statutory Bodies (Discipline And Surcharge) Act 2000 (Act 605)

approved mealtime.

- v. The Hospital must provide a safe working environment to protect staff, visitors, contractors and patients from possible harm and injury.
- vi. Occupational accidents and occupational diseases shall be notified accordingly.

2.4 Management of Assets

These policies will ensure proper recording and accountability for fixed assets and to establish and implement controls necessary to protect the assets of the HPUniSZA.

2.4.1 Acquisition of Fixed Assets

- i. The requirement of a fixed asset shall be decided by the individual department/unit and coordinated by the Finance Department.
- ii. The respective heads of the department shall be responsible for preparing the technical specifications.
- iii. The individual department/unit shall submit the invoice with the supporting documents (including the identification tags form) to the Finance Department to record/register the purchase of fixed assets and process the payment to the vendor.
- iv. The Finance Department shall review the invoice with the supporting documents to indicate approval of recording the fixed assets and initiating the payment process.

2.4.2 Receiving of Donated Fixed Assets

- i. The Medical Social Worker Unit is responsible for receiving donated fixed assets and shall prepare a Receiving Report using *Borang Permohonan bagi Mendapatkan Kebenaran Penerimaan Asset Hadiah* issued by the university for the donation of fixed assets.
- ii. The Medical Social Worker Unit shall submit the Receiving Report to the Finance Department to record the donated fixed assets.

2.4.3 Fixed Assets Identification

- i. The Individual Department is responsible for identifying and tagging all the fixed assets. An identification number shall be assigned and tagged for all fixed assets (other than land and buildings). This tag must have a unique identification number that will be associated with the asset and become part of the asset's record; with tag labels; and shall maintain a list of the used identification codes.

2.4.4 Depreciation of Fixed Assets

- i. The cost of an asset shall be depreciated in value periodically. Depreciation is allocated monthly over the estimated useful life of the capital asset.
- ii. Depreciation is to commence in the first month after the asset is placed into service.

- iii. Fixed assets will be depreciated on a straight-line method over their estimated useful life.

2.4.5 Equipment/Inventory List and Loan

- i. The Hospital shall maintain an up-to-date equipment/inventory list. The department/unit shall also maintain its equipment/ inventory list and the Planned Preventive Maintenance (PPM) schedule.
- ii. Equipment shall not be moved or transferred to outside of the hospital or other institutions without prior approval of the Hospital Director. Any movement of equipment shall comply with the circular of UniSZA.
- iii. For interdepartmental movement of equipment in HPUniSZA, both departments involved must keep clear documentation of the movement.
- iv. The loaning of equipment is limited to items required immediately to ensure the patient's safety and well-being, including equipment used directly and indirectly for patient care. Indirect patient care equipment includes items needed to ensure the smooth, uninterrupted operations of the Hospital.
- v. Department of Business Development & Investment will be in charge of deposit charges for loaned equipment to other institutions.
- vi. Loaned equipment shall be checked before delivery to ensure that it is operational. All returned equipment must be inspected to ensure it is operational.
- vii. Supplies may be loaned to another Hospital but must be replaced with an identical item or an item of equivalent value that is acceptable to the Lender. Delivery and return of the equipment or supplies is the responsibility of the Borrower.
- viii. All loan transactions are recorded in a form *Borang Kebenaran Meminjam/ Membawa Keluar Harta Modal/ Inventori*-maintained by each department; the inventory number, to whom and by whom the loan was made, and the expected date of return to ensure that the privilege of borrowing equipment and supplies is not being abused and that the items are being returned on time.

2.4.6 Disposal of Fixed Assets

- i. Heads of Department shall be responsible to submit a list of equipment to be disposed of/condemned to the Department of Finance and other relevant practice directions and/or circulars issued by UniSZA.
- ii. The Individual Department is responsible for identifying the fixed assets that need to be disposed of and should obtain recommendations from the Department of Engineering and Biomedical.
- iii. The individual department must submit the report together with approval recommendation from the Department of Engineering and Biomedical to the Finance Department.
- iv. The Finance Department shall provide a copy of the approved forms and report that need to be submitted to the Bursary Office to present in

Jawatankuasa Pengurusan Aset Kerajaan (JKPAK) Universiti for final review and approval.

- v. The Finance Department shall be responsible for following up and ensuring the disposing of assets based on the committee's recommendation report and approval.

2.5 Department of Information Technology (JTM)

- 2.5.1 The Hospital shall be responsible to ensure that all Information and Communications Technology (ICT) facilities comply with all standards, policies, existing guidelines of the central agencies such as the Malaysian Administrative Modernisation and Management Planning Unit (MAMPU) and other laws, policies or circulars issued by the government and Hospital from time to time.
- 2.5.2 The Hospital shall ensure that all ICT facilities only be used for purposes related to the function and operation of the Hospital.
- 2.5.3 All ICT services including hardware and software shall be carried out by an appointed company that is accountable to JTM.
- 2.5.4 Maintenance for all ICT facilities shall be carried out regularly according to an agreed schedule. This includes preventive maintenance for both hardware and software.
- 2.5.5 The Hospital shall ensure Business Continuity Plan (BCP) is established and functioning in the unlikely event of an ICT disaster.
- 2.5.6 Official electronic correspondence shall only be used through platforms authorised by the Hospital.
- 2.5.7 All ICT facilities shall be made available to staff in accordance with their responsibilities and privileges written in the Hospital ICT policy.
- 2.5.8 All Users are responsible to comply with ICT policies written in the Hospital ICT policy.
- 2.5.9 The Hospital has the right to take disciplinary action for any breach of the Hospital's ICT policies.
- 2.5.10 This policy is subject to change from time to time.

2.6 Department of Engineering and Biomedical

Department of Engineering & Biomedical shall be responsible for the overall coordination of these services; Cleansing, Linen & Laundry, Clinical Waste Management Services, Biomedical Engineering Maintenance Services and Facility Engineering Maintenance Services.

2.6.1 Engineering and Biomedical Management

- i. A liaison officer for each service shall be appointed to monitor and coordinate all the activities and to ensure compliance with the Concession Agreement (CA), Master Agreed Procedures (MAP), Technical Requirement and Performance Indicator (TRPI) and the Hospital Specific Implementation Plan (HSIP). The HSIP is a dynamic document that shall be reviewed yearly

and may be amended when necessary and endorsed by the Hospital Director.

- ii. The Deputy Director (Clinical Support) shall be the overall coordinator and have regular meetings with the Liaison Officer to discuss issues and remedial actions to be taken to improve the services.
- iii. There shall be a monthly committee meeting to discuss and decide on deductions for non-conformance.
- iv. The Department of Engineering & Biomedical is technically responsible to monitor, evaluate, verify work done and deduction to the Concession Company.

2.6.2 Cleansing Services

- i. Cleansing shall be carried out following the schedule as agreed in the HSIP.
- ii. Cleansing shall be carried out according to the correct technique, suitable equipment and using appropriate detergent.

2.6.3 Linen & laundry

- i. All linen shall be delivered in a manner that provides full protection from contamination during handling and transportation.
- ii. Clean linen already checked and folded to an agreed pattern shall be supplied according to schedule. Linen shall be transported in designated clean or soiled linen carts and shall follow a designated route.
- iii. Supply of clean linen shall be on a top-up basis and comply with the standard of each ward/unit/department/Operation Theatre (OT) as agreed.
- iv. Soiled linen shall be placed in colour-coded bags (Red - infected, Green - OT linen and White - soiled) provided by the concessionaire and collected at the respective areas by the concessionaire as per the agreed schedule.

2.6.4 Clinical waste management Services

- i. Waste shall be handled in accordance with the type of wastes with standard precaution and infection control measures. Types of waste include clinical/medical waste, e-waste, general/solid waste, and recycled waste.
- ii. The transportation of clinical and general waste shall follow a designated route as agreed by Hospital management.
- iii. The chemical waste shall be handled appropriately in accordance with existing acts. All clinical wastes containing infectious and contaminated human tissues, blood, body fluids, excretions, drugs, needles and other related materials shall be treated as scheduled wastes according to existing Laws, Regulations and Practice Guidelines (LRPG).^{[15][16][17][18]}

^[15] Environmental Quality Act 1974 (Act 127) Laws of Malaysia.

^[16] Guidelines on Chemical Management in Health Care Facilities, Ministry of Health, Malaysia, 2010

^[17] Guidelines on The Handling and Management of Clinical Waste in Malaysia, 3rd Edition 2009, Department of Environment, Malaysia, 2009.

^[18] Environmental Quality (Scheduled Wastes) Regulations 2005.

2.6.5 Engineering and Biomedical Maintenance Services

- i. The Concession Company shall be responsible for carrying out Planned Preventive Maintenance (PPM) according to the schedule and procedures recommended by the manufacturer of equipment and the CA.
- ii. The Department of Engineering & Biomedical shall rectify any breakdown (corrective maintenance) within the shortest possible time as specified.
- iii. All departments shall maintain an updated inventory of all equipment and assets in the department. The Heads of Department shall ensure that this equipment is serviced regularly and maintained by the concession company.

2.6.6 Facility Management Maintenance Services

- i. The Concession Company shall prepare training programmes, topics and schedules for future development training plans.
- ii. The Department of Engineering & Biomedical shall confirm training programmes, topics and schedule for future development training plans.
- iii. The Concession Company shall conduct and document training programmes as planned for users. The Department of Engineering & Biomedical shall arrange for users to attend a training programme.
- iv. The Department of Engineering & Biomedical shall establish the detailed scope of work in the Technical Requirements and Performance Indicators (TRPI) document for the Facility Engineering Maintenance Services.
- v. The Concession Company shall confirm the detailed scope of work.

2.2 Security Unit

2.2.1 Security Services

- i. The security services in Hospital may be partly from UniSZA. The scope of services shall be based on the agreed contract.
- ii. The service shall be operated by the UniSZA Department of Occupational Health and Safety (JKKP).
- iii. The security service shall ensure the safety of hospitals and public properties. The Scope shall encompass;
 - Control of the movement of patients, visitors and staff in the Hospital area where only authorised persons are allowed.
 - Ensuring the safety of Hospital assets and properties.
 - Ensuring smooth movement of vehicle traffic in accordance with traffic law.
 - Ensuring physical safety of staff, patients and visitors including appropriate response in the event of risk, hazard or disaster.
- iv. The security plan shall include standard operating procedures including

scheduled patrol, outlet check, visitors check, staff check and 24-hour security location.

- v. The security system shall also include operational procedures in the event of special circumstances such as mass casualty, dignitaries' visits, evacuation, outbreaks and fire. These areas shall be demarcated by the securities personnel. This plan should cover the following items:
 - Safety of the site of evacuation
 - Safety of building left unattended
 - Redirection of vehicle traffic
 - Control of crowd, press and victims and their belongings
 - Ensuring access of authorised personnel to the location
- vi. All security personnel shall be vetted by the police to ensure there are no security personnel with criminal records. Security personnel should undergo a medical examination to ensure they are fit to perform their duties.
- vii. Appropriate technologies may be used such as electronic access cards, security cameras and automatic parking gates.

2.8 Department of Dietetic & Food Serving

- 2.8.1 Production and supply of diet may be carried out by the appointed outsourced food and beverage service company. The food and beverage service company shall be accountable to the Dietetic & Food Serving Department of the hospital.
- 2.8.2 Food shall be prepared according to the Food and Beverage Service Contract Specification prepared by the Hospital.
- 2.8.3 Kitchen facilities and equipment are the Hospital's assets and are rented to the outsourced food service company. Maintenance of equipment shall be done by the appointed Concession Company during the warranty period. After the warranty period, the maintenance of equipment shall be done by an outsourced food and beverage service company. Payment for utilities and rented facilities used by the food services company shall be made to the Hospital according to the contract.
- 2.8.4 Food shall be prepared according to guidelines by the Dietetic & Food Serving Department HPUiSZA.
- 2.8.5 Food preparation shall comply with the Hazard Analysis and Critical Control Point (HACCP), Good Manufacturing Practice (GMP), and *Halal*.
- 2.8.6 The Hospital canteen, cafeteria, and Hospital kitchen shall be regularly inspected to ensure that it meets the standard hygiene requirement by current laws and regulations.^{[19][20]}
- 2.8.7 Food premises in the Hospital are subjected to regular inspections by the in-house Assistant/Environmental Health Officer.

[19] Food Act 1983 (Act 281) Laws of Malaysia.

[20] Food Regulations 1985 (Act 281) Laws of Malaysia

2.9 Operating and Visiting Hours

2.9.1 General

2.9.1.1 The Hospital's operational hours are as follows:

SERVICE	DAY	TIME
Emergency Department	Sunday - Saturday	24 Hours
Specialist Clinics	Sunday - Wednesday	8.00 am - 1.00 pm 2.00 pm - 4.45 pm
	Thursday	8.00 am - 1.00 pm 2.00 pm - 4.30 pm
	Weekends & Public Holidays	Closed
Labour Ward	Sunday - Saturday	24 Hours
In-patient Pharmacy	Sunday - Thursday	8.00 am - 9.00 pm
	Weekends & Public Holidays	9.00 am - 3.00 pm
Out-patient Pharmacy	Sunday - Wednesday	8.00 am - 4.30 pm
	Thursday	8.00 am - 4.15 pm
	Weekends & Public Holidays	Closed

Emergency Pharmacy	Sunday - Thursday	4.30 pm - 8.00 am
	Friday, Saturday & Public Holidays	24 Hours
Daycare Operation Theatre and Endoscopy	Sunday - Thursday	7.00 am - 10.00 pm
Daycare Medical and Paediatric	Sunday - Thursday	7.00 am - 10.00 pm
Emergency OT	Sunday - Thursday	24 Hours
Elective OT	Sunday - Thursday	8.00 am - 5.00 pm
In-patient / Emergency Medical Imaging	Sunday - Saturday	24 Hours
Out-patient Medical Imaging	Sunday - Wednesday	8.00 am - 1.00 pm 2.00 pm - 4.45 pm
	Thursday	8.00 am - 1.00 pm 2.00 pm - 4.30 pm
	Weekends & Public Holidays	Closed

2.9.1.2 Visiting hours shall be determined by the Hospital management depending on the current health situation such as during the pandemic or any other government direction or order. Generally, the visiting hours shall be as follows:

DAY	TIME
Weekdays	12.30 pm - 2.00 pm 5.00 pm - 7.00 pm

Friday, Saturday & Public Holidays	11.00 am - 2.00 pm 5.00 pm - 7.00 pm
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- 2.9.1.3 Special visiting hours apply to critical care areas such as the CIC, NICU and SCN. The visiting hours shall be as follows:

DAY	TIME
Weekdays	1.00 pm - 2.00 pm 5.00 pm - 7.00 pm
Weekends, Friday, Saturday & Public Holidays	1.00 pm - 2.00 pm 5.00 pm - 7.00 pm

- 2.9.1.4 During visiting hours, visitors shall be allowed to visit patients in the general wards except in the event of a pandemic.
- 2.9.1.5 Visits to the critical care areas shall be restricted to two visitors per patient at any time.
- 2.9.1.6 Children aged below 12 are not allowed to visit patients in the Hospital.

2.9.2 Outside Visiting Hours

- 2.9.2.1 There shall be no visitors outside visiting hours unless special permission is given by the doctor in charge. This visit shall not exceed more than half an hour. All visits after visiting hours shall be recorded.
- 2.9.2.2 A relative may be allowed to accompany a patient subject to the approval of the ward staff. A special pass (*Pas Menunggu*) shall be issued to one person in the following situation:
- Relatives who are allowed to accompany critically ill and bedridden patients.
 - Only same gender relatives shall be allowed to accompany a patient after 9.00 pm.
 - The mother or guardian is allowed to accompany a child in the paediatric ward.
 - The mother of a baby admitted to the Special Care Nursery for breastfeeding purposes.

2.9.3 Other Hospital Visitors

- 2.9.3.1 Registered Hospital volunteers shall be allowed to enter the Hospital up to 9.00 pm.
- 2.9.3.2 Members of the Board of Visitors with identification cards shall be allowed to enter the Hospital at any time for formal duties.
- 2.9.3.3 Very Important Persons (VIPs) on official visit shall be accompanied by the Hospital staff.

2.10 Corporate Communication and Customer Services Unit (CCaCS)

2.10.1 Information Counter

The information counter service is open every day from 8.00 am to 4.45 pm (Sunday to Wednesday) and from 8.00 am to 4.30 pm (Thursday) except for public holidays.

Information counter services provide:

- i. Information and general guidelines related to treatment provided at HPUniSZA including treatment charges, visiting hours, referral cases, Department/Unit locations, wards and clinics.
- ii. Help for patients who seek help.

2.10.2 Customer Service

Manage complaints/recommendations/compliment

- i. All complaints/grievances related to patient care should be managed according to the established Work Instructions.
- ii. All complaints/suggestions/acknowledgments must be responded to within 1 day from the date of receipt.
- iii. Verbal or non-verbal complaints are accepted. Complaints are received through various mediums. Complaint details will be logged into the complaint system.
- iv. Situations that have the potential to harm the patient and the organisation will be reviewed immediately.
- v. All complaints including complaints made by third parties on behalf of patients, staff or any customers of HPUniSZA must be directed to and handled by the Corporate Communication and Customer Services Unit (CCaCS).

2.10.3 Release of Information

- i. The Director of HPUniSZA or his representative, the Deputy Director or the Public Relations Officer (PRO) must be authorised to make statements to the press on the condition that they are consulted and approved by the UniSZA

Corporate Communications Center (CCORM).

- ii. For ethical and legal reasons, all hospital staff shall respect the confidentiality of information, obtained either directly or indirectly, relating to any patient, his medical condition, diagnosis and treatment according to the Personal Data Protection Act (PDPA).
- iii. As the release of patient information may have serious implications, any member of staff who does not comply with this policy will be subject to disciplinary action.

2.10.4 Press and media releases

- i. All inquiries from newspaper reporters, television stations or other news media and social media should be referred to the Corporate Communication and Customer Services Unit (CCaCS).
- ii. Reporters, film crews and photographers at HPUniSZA premises must always be accompanied by a Public Relations Officer or staff/nominee to ensure the confidentiality, privacy and safety of patients and families are protected.
- iii. The hospital spokesperson shall be permitted to release additional patient information for publicity purposes, after written consent is obtained from the patient/surrogate.

2.10.5 Multimedia production

- 2.10.5.1 Requests for photos, videos, audio recordings or teleconferences by HPUniSZA staff and Faculty, news media, television production crews or others for the purpose of teaching, publishing or broadcasting to the public, should be referred to the Public Relations Officer and permission will only be granted by the HPUniSZA Director or Deputy Director.
- 2.10.5.2 Any video, photo or film produced at HPUniSZA must be for teaching purposes only and become the property of HPUniSZA.
- 2.10.5.3 Permission for the privilege of photographing/interviewing patients in the hospital may be granted if:
 - a) In the opinion of the doctor in charge of the case, the patient's condition will not be endangered, and
 - b) The patient (or in the case of a child, a parent or guardian) has consented to be photographed/interviewed.
 - c) For all purposes above in 5.2 and 5.3, written consent must be obtained from the patient or surrogate. (Refer to Consent Form for Audio Visual Recording by Patient/Surrogate).

2.10.6 Photography/Filming/Interview

- 2.10.6.1 As recordings made for clinical purposes constitute part of a patient's

medical record, they should be treated in the same way as any other part of the medical record.

- 2.10.6.2 Prior consent must be obtained if the practitioner is planning to take clinical photographs or audio-visual/multimedia recordings. (Refer to MMC guideline).^[21]
- 2.10.6.3 Any media shall not be allowed to make any recording and interview with anyone including staff, patients and visitors in the Hospital without consent from the Hospital Director.
- 2.10.6.4 Any commercial filming or drama shooting in the Hospital compound shall not be allowed without consent from the Hospital Director.

2.10.7 Public Forum and Exhibition

- 2.10.7.1 The Hospital may organise talks or exhibitions to provide health education to the public.
- 2.10.7.2 Health promotional activities may also be organised to create public awareness and encourage public participation.

2.10.8 Event management

- 2.10.8.1 Managing official events hosted by HPUiSZA.
- 2.10.8.2 Cooperation with Departments, Non-Government Organisation (NGOs), corporate bodies and others in conducting official events involving HPUiSZA held inside or outside HPUiSZA.
- 2.10.8.3 Holding press conferences, press releases, interviews (newspapers, broadcasts and magazines), filming and documentaries.

2.11 Communication System

2.11.1 Telephone

- The Hospital uses a Private Automatic Branch Exchange (PABX) telephone system for in-hospital and out-of-hospital calls.
- There are three types of PABX telephone lines in the hospital. Type A-line allows calls to be made internally and externally including cellular lines. Type B-line allows calls to be made internally and externally excluding cellular lines. Type C-line can only make internal calls.
- Type A-line shall be made available for The Hospital Director as head of the organisation.
- Heads of the Clinical Department shall be provided with a Type B-line. Specific areas shall also be provided with a Type B-line. All other telephone lines within the Hospital shall be of Type-C line.
- Telephones shall be for official use only. The usage of telephone will be monitored by the operators.

^[21] Malaysian Medical Council Guideline. Reference - <https://mmc.gov.my/laws-regulations/>

- A two-way communication system shall be provided in the ambulances for communication between the ambulance and the base station in the Emergency and Trauma Department (ETD).

2.11.2 Nurse Call System

- A nurse call system shall be provided to all beds for patients to use when assistance is required. The system may be extended to patient areas such as washrooms and toilets.
- Nurses shall attend to the patient as soon as possible when the nurse call system is activated.

2.11.3 Public Address System

- The Public Address (PA) system shall be used to make announcements, alerts and provide information.
- The PA system may be used for emergencies using specific codes as stated in the Hospital Disaster Preparedness Policy.

2.11.4 Social Media

- Communication and the use of official social media shall at all times adhere to the current technology in various social media applications and shall be adopted as means of laws, regulations and circulars issued by UniSZA subject that they were issued from UniSZA's official channels.

2.11.5 Short Message Services (SMS)

- Notifications through SMS including but not limited to an appointment, scheduling and patient queue shall be adopted as a means of communication.

2.12 Transport Unit

2.12.1 General Transport Unit and Ambulances

2.12.1.1 The Hospital shall provide ambulance services for patients and transportation for both patients and staff. Ambulances and vehicles shall always be well-maintained and ready for use.

2.12.1.2 Hospital vehicles shall be used for specified purposes as follows:

- Ambulances shall be used for pre-hospital care and inter-hospital transportation of patients.
- Vans shall be used to transport supplies and materials.
- Mobile transport shall be used to transport staff and ambulant patients.
- Cars shall be used to transport staff.
- A lorry shall be used to transport bulky items such as furniture and equipment.

- Patient Transport Service Vehicle (PTSV) shall be used for stable patient transportation to rescue medical care as necessary.
- Mobile Medical services vehicles such as mobile mammogram etc shall be used for its specific purposes.

2.12.1.3 Hospital vehicles shall be driven by Hospital drivers with valid driving licences and shall always abide by the road traffic rules and regulations at all times.

2.12.1.4 Ambulance drivers are required to undergo specific training.

2.12.1.5 The ambulances shall be under the responsibility of the ETD while the other vehicles will be under the responsibility of the transport unit supervised by the Administration Department.

2.12.1.6 Relatives are not allowed to accompany patients in the ambulance and are required to sign an indemnity form if they do. However, parents/carers shall accompany paediatrics patients.

2.12.1.7 The occupancy of the vehicles shall be in accordance with the manual of each type of vehicle.

2.12.1.8 The usage of the appropriate transport during an emergency is under the discretion of the Hospital Director or Deputy Director.

2.12.1.9 The logbook of all vehicles and ambulances shall be updated regularly.

2.12.1.10 Drivers shall ensure regular cleaning of the vehicles and ambulances.

2.12.2 Pneumatic Tube System

2.12.2.1 The pneumatic tube shall be used to transport pathology specimens, medicines, documents and medical records, with weight according to the system specification. Items other than this are not allowed.

2.12.2.2 Items shall be placed in the special container provided before being transported in the pneumatic tube.

2.12.2.3 The department using the pneumatic tube (sender) shall be responsible for the proper and safe transfer of items and to trace the items in case of delay or non-arrival at the receiving end.

2.13 Traffic Control

2.13.1 The Hospital shall implement a traffic system within the Hospital to avoid traffic congestion. Route to the Emergency Trauma Department (ETD) shall only be used by ambulances and public/private vehicles bringing emergency cases, for exit and entry.

2.13.2 Drop-off and pick-up zones may be provided near the entrance to the Main Lobby, Specialist Clinics, Obstetrics and Gynaecology Assessment Centre (OGAC), ETD, Rehabilitation Medicine Department, and *Surau* for patient's convenience.

2.13.3 Parking outside the designated parking areas shall be strictly prohibited.

Chapter 3:

Hospital Governance

3.1 Board of Visitors

3.1.1 Hospital Board of Visitors

- i. The Hospital shall establish a Board of Visitors as required by the Ministry of Health (MOH). The Board members shall be appointed by the *Lembaga Pengarah Universiti* (LPU) and appointed members shall be provided with an identification card.
- ii. The Board of Visitors shall act as a link between the Hospital and the community.
- iii. The Board members shall be briefed on the Hospital's organisational structure and services, the rules and regulations and the Board of Visitors roles and responsibilities.
- iv. Board members shall be allowed to make visits to the wards and other public areas during or after office hours. The Hospital management shall take appropriate actions on the feedback received or issues raised by the Board.
- v. Board members shall be invited to attend Hospital functions and activities including the relevant Continuous Medical Education (CME) session.

3.1.2 Hospital Volunteers

- i. An individual or group who wish to be the Hospital volunteers may apply to the Hospital and shall follow the procedure required for approval.
- ii. A Medical Social Work (MSW) Officer or any officer from the Hospital may be appointed as a coordinator. This person will be in charge of guiding the volunteers and monitoring their services.
- iii. The Hospital volunteer shall abide by the Hospital rules and regulations and shall professionally render services.

3.2 Hospital Safety and Security

3.2.1 General Security

- i. The areas in the Hospital shall be identified either as high, medium or low-security risk. Examples of high-security areas are the entrances, stores, revenue units, wards and delivery suites.
- ii. Areas identified as high or medium security risk shall have security measures installed and security guards placed at all times. Other areas shall have regular site patrols by the security guards.
- iii. Clear 'No Entry' signs shall be placed in areas and on doors to the rooms which are restricted for the staff or authorised personnel only.
- iv. The department/unit heads are responsible for the security procedures within the department and staff compliance with the procedures.
- v. Specific areas (as decided by the Hospital Director) shall have electronic surveillance and be centrally monitored.

- vi. Entrance to all wards and departments shall be equipped with a mechanical control system or electronic system. Prisoners who are in-patients must be guarded at all times by prison wardens, subject to law and regulation.^[22] The Hospital shall take full responsibility for the prisoners' custody during their stay in the Hospital.
- vii. Patients who are subjected to police cases and prisoners must be accompanied by the relevant authority at all times in the Hospital.
- viii. Subject to law and regulation, controlled drugs shall be stored in a Dangerous Drug Act (DDA) cupboard.^[23]

3.2.2 Occupational Safety and Health (OSHA)

3.2.2.1 Fire Safety

- i. The Hospital shall appoint a Safety Officer and have a fire contingency plan in place.
- ii. Appropriate fire equipment shall be made available in all areas and fire equipment regularly maintained.
- iii. The person in charge of the respective areas shall ensure regular inspections are carried out on all firefighting facilities, fire retardant doors and escape routes. The person shall also be responsible for the fire safety procedures and ensure that staff adhere to these procedures.
- iv. Fire retardant doors are to be kept closed at all times but not locked. If exit doors need to be locked, the keys shall be made readily available.
- v. In the event of a fire, patients shall be evacuated following the principle of horizontal evacuation and if the fire continues to spread, to move vertically down.
- vi. All staff shall receive training on fire safety, evacuation procedures and the use of firefighting equipment. Fire drills shall be conducted regularly, at least once a year.

3.2.3 Radiation Protection

- i. The Hospital shall establish a Radiation Protection Committee (RPC) chaired by the Hospital Director/Appointed Chairman.
- ii. The Hospital shall appoint a Radiation Protection Officer (RPO) to oversee and coordinate activities related to radiation protection.
- iii. Policies, procedures and guidelines on radiation safety and protection shall be made available to all relevant departments/units.
- iv. Briefing on the policies and procedures on radiation safety and protection shall be conducted for specific staff.

^[22] Section 40(1) of the Prison Act 1995 [Act 537] Laws of Malaysia.

^[23] Dangerous Drugs Act 1952 [Act 234] Laws of Malaysia.

- v. The radiation exposure of staff is routinely monitored including necessary blood count and medical examination.
- vi. Staff who carry out full time X-ray duties and are exposed to X-ray radiation are eligible for a special medical leave of 14 days in one year.
- vii. Facilities and imaging personnel shall adhere to regulations and guidelines on the use of ionising radiation.^[28]
- viii. For women of childbearing age, the guidelines by the MOH shall be adhered to.^[29]
- ix. X-ray equipment shall be operated by a radiographer.

3.2.4 Infection Control

- i. The Hospital Infection and Antibiotic Control Committee (HIACC) functions as a governance body to coordinate hospital activities in the prevention and control of Healthcare Associated Infection (HCAI), promote the judicious use of antimicrobials and control the emergence of Antimicrobial Resistance (AMR). The HIACC involves multi-disciplinary participation from various departments and units.
- ii. The Hospital Infection and Antibiotic Control Committee (HIACC) shall be chaired by an Infectious Disease Consultant or Clinical Microbiologist assisted by an Infection Control Doctor and Infection Control Nurses.
- iii. The HIACC may conduct the following:
 - Formulate and review policies and procedures related to infection control, AMR and antibiotics usage in the hospital
 - The Committee reviews issues related to infection control and antibiotic use and advises the Healthcare Workers (HCWs) through the Head of Department/Unit.
 - Disseminate knowledge, improve skills and inculcate desired values in health care workers about the subject through education and training.
 - Disseminate and ensure compliance with the policies and procedures among health care workers and (where applicable) patients, relatives and visitors.
 - Plan out Hospital-wide infection control programs and activities yearly. This function is incorporated into the day-to-day activities of personnel, patients and visitors.
- iv. The ward Nurse Manager or senior staff nurse shall be the infection control link nurse for the respective ward/unit/department.

^[28] Atomic Energy Licensing Act 1984 (Act 304) LPTA, BSS 1988, MS 838).

^[29] Radiology Information System, Health Informatics Standard, MOH, KPK (2) dlm. KKM-153 (13/3) Bhg. 9 1998.

- v. All Infection control nurses shall have Ministry of Health Post basic training.
- vi. The infection control nurse/personnel are full-time members of the hospital's infection control team.

Chapter 4:

Clinical Governance

4.1 Patient-related Policies

The Hospital shall at all times observe the rights of patients and deliver service to patients in accordance with the laws and regulations issued by the Malaysian Medical Council which includes but not limited to the following.^[21]

A. Ethical Codes:

- i. Code of Professional Conduct
- ii. Good Medical Practice
- iii. Credentialing & Privileging
- iv. Expert Witness
- v. Standing Orders for the Conduct of Inquiries at the Preliminary Investigating Committee Level and at the Council Level (Medical Act 1971)
- vi. Standing Orders for the Conduct of Inquiries of the Malaysian Medical Council (Medical Act 1971 (Amended 2012))

B. Ethical Guidelines:

- i. Guidelines on Consent
- ii. MMC Guideline on Plagiarism
- iii. A Guidebook for House Officers
- iv. Guidelines for Medical Practice for Doctors Beyond the Age of 70 Years
- v. Guidelines On the Ethical Aspects of Aesthetic Medical Practice
- vi. Confidentiality Guidelines
- vii. MMC Guidelines for Good Dispensing Practice
- viii. Brain Death
- ix. Clinical Trials & Biomedical Research
- x. Good Clinical Practice
- xi. Dissemination of Information by the Medical Profession
- xii. Ethical Implications of Doctors in Conflict Situations
- xiii. Medical Genetics & Genetic Services
- xiv. Medical Records & Medical Reports
- xv. Organ Transplantation
- xvi. Relationship between Doctors & the Pharmaceutical Industry
- xvii. Assisted Reproduction

^[21] Malaysian Medical Council Guideline. Reference - <https://mmc.gov.my/laws-regulations/>

- xviii. Stem Cell Research & Stem Cell Therapy
- xix. Managing Impaired Registered Medical Practitioners
- xx. Audio & Visual Recordings
- xxi. Restriction on Practice of Ozone Therapy & Chelation Therapy
- xxii. MMC's Position on Managed Care Practice
- xxiii. Human Immunodeficiency Virus (HIV) & Blood Borne Virus Infections Guidelines

4.1.1 Patients' Rights

- i. A patient has the right to receive a holistic approach to patient care, including health promotion, disease prevention, diagnosis, treatment, disease-management, rehabilitation and palliative care services.
- ii. Information regarding the relevant policies, rules and regulations of the Hospital shall be made available including quality and performance improvement of the Hospital.
- iii. No patient shall be discriminated against race, gender, sex, religious belief, social and economic status or any other factors.
- iv. The Hospital shall respect the patients' rights inclusive of the cultural, spiritual and religious beliefs of the patient and families.
- v. The Hospital shall be responsible for the safety of the patient during the Hospital stay.
- vi. Medical care shall be given in a considerate and respectful manner and information about the medical care be kept confidential.
- vii. The Identity of the healthcare providers and caregivers involved in the patients' treatment shall be made known.
- viii. Treatment shall be given based on the patients' clinical condition. Treatment provided to patients is individualised for the disease and given safely.
- ix. Healthcare providers shall listen and respond promptly to a patient's report of pain and manage pain appropriately.
- x. The Hospital shall communicate with the patient and family on the disease condition and the treatment options available including the right to a second opinion.
- xi. The patient and family will be involved in all decision making. Any decision made by the patient and family shall be respected including refusal of treatment and Discharge Against Medical Advice (DAMA).
- xii. Appropriate counselling shall be given to the patient and family members prior to DAMA.
- xiii. Relevant information on the organ procurement, donation and transplantation process shall be made available where necessary.

- xiv. The Hospital shall make available the approximate cost of treatment to the patient prior to the provision of care and provide an itemised statement of charges upon discharge. Information regarding financial and other assistance that may be available will be provided to the patient.
- xv. Patients with hearing/speech impairment and language barrier have the right to request an interpreter.
- xvi. Children, disabled individuals, the elderly and other populations at risk have the right to receive appropriate protection while receiving treatment at the Hospital. This includes comatose patients and individuals with mental or emotional disabilities. Such protection extends beyond physical injury to other areas of safety such as protection from abuse, negligent care, and withholding of services or assistance in the event of an evacuation. The above group of patients will be provided special amenities to meet their needs, e.g. special lanes, disabled-friendly toilets, children's toilets and sink, etc.
- xvii. A patient and his/her family shall have access to an appropriate grievance mechanism (*Mekanisma Penyelesaian Rungutan/Complaint*).
- xviii. The Hospital shall inform patients and families about its grievance mechanism to receive and act on complaints, conflicts and differences of opinion about care and the patients' right to participate in these processes.
- xix. Appropriate and relevant consent shall be obtained from patients who are involved in medical research, investigations or clinical trials. Patients and families will be given the following information:
 - Expected benefits
 - Potential risks and complications
 - Procedures that shall be followed
 - Obtaining informed consent
 - Withdrawal from participation
 - Suspension or termination of the research

4.1.2 Patient's Responsibilities

A patient shall:

- i. Be cooperative and participate in the treatment and medical care provided by the Hospital.
- ii. Observe appointments and treatment programmes provided by the Hospital.
- iii. Provide accurate and complete information to the Hospital including the use of traditional medicines.
- iv. Furnish payments of Hospital bills including any costs associated with

treatment and services received in the Hospital.

- v. Be responsible for the patient's own informed decisions.
- vi. Maintain personal conduct and behaviour at all times so as not to interfere with the well-being of other patients or providers of healthcare.
- vii. Be responsible to maintain his/her health.
- viii. At all times, abide by the rules and regulations of the Hospital.
- ix. Treat all Hospital staff, other patients and visitors with courtesy, respect, mindful of noise levels, privacy and number of visitors.
- x. Speak up and ask questions when he/she does not understand information or instructions.
- xi. Be fully responsible for outcomes if the patient does not follow the care and treatment plan provided by the Hospital.
- xii. Be considerate of hospital facilities and equipment and use them in such a manner so as not to abuse them.

4.1.3 Consent

- i. Every patient has a choice whether or not to undergo a proposed procedure, surgery, treatment or examination. Informed consent usually refers to the idea that a person must be fully informed and understand the potential benefits and risks of their choice of treatment. An uninformed person is at risk of mistakenly making a choice not reflective of his or her values or wishes.
- ii. Obtaining a patient's consent is a specific legal requirement for certain procedures and is a part of good medical practice.
- iii. The consent shall be obtained from the patient or next of kin prior to carrying out any clinical procedure. The consent shall be obtained from the patient if he/she is 18 years old or more and if he/she is physically and mentally competent.
- iv. For patients below the age of 18 years, consent shall be obtained from the parent or legal guardian.
- v. All consent must be taken by a registered Medical Officer (MO) or Specialist performing the procedure using the consent/appropriate form. The communication includes but not restricted to:
 - Patient's condition
 - Proposed treatment/ procedure
 - Potential benefits and risks
 - Likelihood of success/failure
 - Possible alternatives
 - Possible problems related to recovery
 - Possible results of non-treatment

- vi. Regarding consent in a situation of a life-saving patient where all efforts to trace relatives and next of kin have failed, the Hospital Director should be informed and another two clinical specialists from the related discipline may give consent for the procedure to be carried out. The consent and efforts made to trace the relatives/ next of kin shall be documented in the case notes.

Next of kin refers to: ^{[53][54][55][56][57][58][59][60][61][62]}

- a) Husband or Wife
 - b) Son or Daughter above 18 years old
 - c) Mother or Father
 - d) Brother or Sister
 - e) Grandparent
 - f) Grandchild
 - g) Maternal or Paternal Uncle or Aunt
 - h) Nephew or Niece
- vii. For patients having mental illness who is required to undergo surgery, electroconvulsive therapy or clinical trials, consent for any of them may be given by:
- The patient himself if he/she is capable of giving consent as assessed by a psychiatrist
 - His/her guardian, in the case of a minor
 - A relative in the case of an adult, if the patient is incapable of giving consent
 - Two psychiatrists, one of whom shall be the attending psychiatrist may give consent if there is no traceable guardian or relative of the patient and the patient himself/ herself is incapable of giving the consent
- viii. For a patient below the age of 18 years who requires medical treatment, consent shall be obtained as follows:
- If in the opinion of a treating doctor, the patient requires surgery or psychiatric treatment due to serious illness, injury or condition, the

[53] Gale Encyclopaedia of Public Health: Informed Consent, 2012.

[54] Guardianship of Infants Act 1961.

[55] The Law Reform (Marriage & Divorce) Act 1976.

[56] United Nations Convention on the Rights of the Child, 1989.

[57] Mental Health Act 2001.

[58] Milwaukee Journal Sentinel on Paul G Gebhard, 1997.

[59] Eric Pace, the New York Times, on P.G. Gebhard, 26 Aug 1997.

[60] Private Healthcare Facilities and Services Regulations 2006.

[61] The Malaysian Medical Council Ethical Guideline on Audio & Visual Recordings.

[62] Human Tissues Act 1974.

consent shall be given by the parents/guardian of the child/any persons having authority to consent for the treatment;

- If the treating doctor has certified in writing that there is an immediate risk to the health of a child and medical/surgical/psychiatry treatment is necessary, a protector may authorise without obtaining consent from the parents/guardian of the child/any persons having the authority, but only under any of the following circumstances: -
 - a. That the parents/guardian of the child/any persons having the authority to consent to the treatment has unreasonably refused to give or abstained from giving consent to such treatment;
 - b. That the parents/guardian of the child/any persons having the authority to consent is not available or cannot be found within a reasonable time.
 - c. The Protector believes on reasonable grounds that the parents/guardian/the authorised person has ill-treated, neglected, abandoned or exposed, or sexually abused the child. According to Child Act 2001, Protector is defined as the Director-General Ministry of Health, the Deputy Director-General Ministry of Health, a Divisional Director of Social Welfare, Department of Social Welfare, the State Director of Social Welfare of each of the State, and a Social Welfare officer appointed.
- ix. Separate consent must be obtained from the patient or next of kin when body parts or organs are taken for academic or research purposes.

4.1.1 Counter Services

- i. The Hospital shall have a general information counter and dedicated counters e.g. registration counters, clinic counters, ward counters etc. and the functions include:
 - Providing information
 - Providing assistance
 - Receiving suggestions or complaints
- ii. The counter shall be manned by competent persons.
- iii. A senior staff shall supervise the effective delivery of related counters.
- iv. All counters shall be operational according to a determined schedule.
- v. Priority Lane at the registration counters shall be provided to the following clients (except in ETD):
 - Children aged two years and below
 - Senior citizens (60 and above)
 - Blood donors (according to existing guidelines)
 - Disabled persons (*Orang Kelainan Upaya*)

- Persons in custody (*Orang Kena Tahan*)
- Organ donors

4.2 Appointment and Scheduling

- Appointments may be made by phone, email, website, application, letter or personally at the clinic.
- Appointments will be booked in the Total Hospital Information System (THIS) by the clinic at the registration counter.
- Services shall be given on an appointment basis except for the ETD.
- Early rescheduling of appointment shall be upon approval by the relevant department or according to the policy of the department/unit as approved by the Hospital management.
- Patient appointment rescheduling should be done by informing the department clinic through phone or website.
- All clients shall be informed of the relevant document/item to facilitate the registration process e.g. ID Card/passport, referral letter, appointment card and Letter of Guarantee (e-GL)/Pensioner Card etc.
- Appointment date shall be given and booked in THIS system by the respective department upon referral by other departments during or after office hours.

4.3 Registration

- Each patient shall have only one Running Number (RN) for personal identification.
- Children below the age of 12 years shall be registered using MyKid/Birth Certificate/Passport.
- Registration format shall be as specified by the Hospital management. The staff at the registration counter shall be responsible for ensuring the completeness of the information and no duplication.
- All clients requiring registration must present relevant documents at the designated registration counters.
- Without proof or evidence of the method of payment (Guarantee Letter/ Exemption/ Pensioner/Insurance etc.) a patient will be classified as a self-paying patient.
- Identification of a newborn shall use his/her mother's Mykad/passport/other ID number plus prefix "D" followed by a sequence of delivery. For example, a second baby born to a mother with Mykad number: 800417035646 shall be identified as 800417035646 D02.
- All newborns shall be allocated with an RN and be registered. All stillbirths (fresh/macerated) shall be allocated with an RN and be registered for clinical/reporting/costing reasons.
- Registrations for: Birth Before Arrival (BBA)/Birth in Ambulance (BIA)/Birth in other vehicles:

- The transporting ambulance shall reroute to the nearest health facility and registration shall be done at the facility (public/private).
 - If the umbilical cord is cut by Hospital staff, it is considered a Hospital birth.
 - In Hospital statistics, the location of this birth is considered BBA with the name and Mykad of staff attending to the delivery recorded.
- ix. In circumstances whereby a patient is already registered at In-patient Registration, and deteriorates during the journey to the ward or OT, there will be two possibilities:
- If the patient is still near the vicinity of ETD, the patient shall return to ETD, and if death occurs, it shall be an ETD statistic death. The admission shall be cancelled in the system with justification.
 - If the patient is nearer to the ward, he/she shall be wheeled to the ward, and it will be the ward statistic death if the patient succumbs.
- x. Registration for an unknown individual is processed as a normal patient with temporary identification/information given. Information will be updated once verified details are given by the patient/next of kin. The format for temporary information of an unknown patient is as follows:
- Name: Unknown or XYZ
 - Gender: Male/Female
- xi. If information of an unknown patient is not available, a police report shall be made.
- xii. During system downtime, manual pre-printed forms must be used for registration purposes. After downtime, all manual registration shall be keyed into the system.

4.4 Consultation

- i. The Hospital shall ensure the safety, confidentiality and privacy of the patients throughout consultation and examination.
- ii. Patients at the Specialist Clinic shall be managed by the doctor relevant to the particular illness/specialty.
- iii. The assessment of patients shall be documented in the designated clerking forms comprising a full medical history and physical examination.
- iv. Nursing assessment and entries by other allied health personnel shall be documented in the patient's case notes as integrated case notes.
- v. Examination of a patient by a doctor of a different gender must be done in the presence of a chaperone. Examination of a child must be accompanied by parents or caretaker.
- vi. As a teaching hospital, all students may be allowed to be present during the consultation.

4.5 Admission

4.5.1 Patient Admission Flow

- i. The patient's admission formalities shall be carried out by the Admission Unit.

- ii. Stable patients from ETD and any referring hospital/health clinic can be admitted directly to the relevant ward after consultation with the ward doctor/Specialist on call.
- iii. All unstable patients shall be optimised in the ETD before admission to the ward. Direct admission to the Intensive Care for the very ill patient shall be arranged with prior consultation and agreement by the anaesthetist/intensivist in charge.
- iv. Maternity cases (24 weeks and above) shall be sent directly to the Obstetrics and Gynaecology Assessment Centre (OGAC) and the necessary admission formalities attended to subsequently.
- v. Patients at specialist clinics requiring admission MUST be directly admitted to the ward and must NOT be via ETD except for Psychiatry Clinic.
- vi. Patients or their relatives shall pay a deposit or produce a Letter of Guarantee on admission in accordance with the fees schedule.
- vii. Patients shall be transported on mobile beds, transport trolleys (incubator/cot bed/bassinet) or wheelchairs escorted by medical staff.
- viii. The ward/department shall be responsible for transporting/accompanying patients within the department as well as to other departments.
- ix. Admission of a patient to a specific ward shall be withheld if/when the ward is temporarily gazetted as an infectious disease ward during an outbreak of infectious disease.

4.5.2 Bed Management

- i. Overall management of beds shall be the responsibility of the Bed Management team headed by the Department of Nursing.
- ii. There shall be a Bed Management Committee to oversee the mobilisation of beds, especially during access blocks.
- iii. Paediatric patients below 12 years of age shall be admitted to the Paediatric ward.
- iv. No allowance shall be made for additional or temporary beds in the wards. In the event where the ward is full, patients are temporarily admitted to an appropriate or related ward until the availability of a bed.
- v. If a patient cannot be admitted to a designated ward due to the unavailability of a bed, the patient can be lodged to another ward except to Obstetrics and Gynaecology (O&G) and Paediatric Wards.
- vi. If no beds are available in the wards, arrangements shall be made to transfer the patient to another hospital by the respective primary team.
- vii. If O&G and Paediatric wards are full, O&G and Paediatric patients cannot be lodged to other wards and arrangements shall be made to transfer patients to another hospital.
- viii. Discharged patients shall vacate the bed on the same day of discharge.

Discharged patients that meet the criteria will be transferred to the discharge lounge.

- ix. Bed swapping and transfers shall be documented in the Electronic Medical Record (EMR) by the ward staff.

4.5.3 Admission of Unknown Patients (comatose, psychiatric, amnesic, etc.)

- i. All available information pertaining to the unknown patient admitted shall be documented in the admission book as “unknown” and a registration number or RN shall be given.
- ii. All valuables belonging to these groups of patients shall be kept in an envelope sealed, named, dated and witnessed by 2 medical personnel.
- iii. The police shall be notified immediately and re-notified if the patient remains unidentified after 24 hours.

4.5.4 Admission to Very Important Person (VIP) or Executive Ward

- i. The decision to admit a patient to the VIP or Executive Ward shall be determined/verified by a Specialist according to the clinical condition.
- ii. When a patient’s clinical condition becomes unstable and requires intensive care, the patient shall be transferred to High Dependency Unit (HDU)/Intensive Care Unit (ICU)/Cardiac Care Unit (CCU) and that bed shall be vacated. Patients on the waiting list can then be admitted to occupy the bed.
- iii. When there is no available bed in the VIP or Executive Ward, the patient shall be admitted to the general ward and placed on a waiting list for the VIP or Executive Ward. The transfer shall be made when a bed is available.
- iv. Admission of Royalties/Very Very Important Person (VVIPs)/VIPs shall be based on the respective state/national protocol.

4.6 In-Patient

4.6.1 In-patient Ward

- i. An identification wristband shall be provided to all in-patients and must be worn at all times during the Hospital stay.
- ii. Assignment of beds shall be done by the team leader of the ward. Patients of the same gender shall be admitted to the same room or cubicle.
- iii. The attending nurse shall inform the doctors within 15 minutes of a new patient being admitted; stable patients shall be seen by a doctor within 60 minutes.
- iv. An individual patient shall be provided with a bed, chair, locker and Hospital garments. Facilities like toilets, baths and rest areas shall be shared.
- v. All patients/caretakers shall be given an orientation on the Hospital which includes information about housekeeping, ward and Hospital facilities, plus safety instructions by the ward staff.

- vi. Patients are advised against wearing jewellery or bringing along valuable items including a large amount of cash for admission. The patient shall be advised to immediately hand over the valuables to the next of kin to bring home for safekeeping. The Hospital will not be responsible for any lost item.
- vii. In-patients shall be reviewed at least twice a day by a MO and once a day by a specialist. Additional reviews when necessary will be according to the patient's clinical condition.
- viii. The use of cell phones is permissible, however charging will be done at designated areas.
- ix. Patients and/or relatives are not allowed to take pictures or videos in the ward.

4.6.2 Seriously ill Patients/Death in Line (DIL)

The specialist in charge of all patients deemed seriously ill shall be responsible for communicating this information to the relatives/next of kin in a tactful manner that is clearly understood by them. The details of the discussion shall be documented in the patient's case note.

4.6.3 Do Not Resuscitate (DNR)

- i. If a patient is deemed seriously ill, a discussion regarding the goal of care shall be made between the Consultant/Specialist in charge from the primary team and Consultant/Specialist from other disciplines involved in the care of the patient with the patient's family.
- ii. The goal of care discussed shall be explained to the patient's family and agreed upon.
- iii. If the decision to "Do not resuscitate" has been agreed upon, it must be documented and signed by the consultant/specialist in the patient's case note and the DNR form be completed.

4.7 Discharge

All discharges shall have a Discharge Summary which includes diagnosis, medication and management plan. The International Classification of Disease (ICD) 10 coding for diagnosis and ICD 9-CM for a procedure should be stated upon discharge. Every patient shall be issued a bill of service at discharge.

A discharge package (summary, bills, medications, Medical Certificate (MC), follow-up appointment, health information, patient experience survey etc.) shall be given to the patient prior to discharge.

4.7.1 Planned discharge

- i. The Medical Officer (MO)/Specialist in charge of the patient shall be responsible for communicating information about planned discharge at least 24 hours prior to the event.
- ii. Identification wristbands for adult patients shall be removed in the ward upon discharge.

- iii. Identification wristbands for newborn and paediatric cases shall be removed by the auxiliary police/security at the Hospital lobby.
- iv. Ward nurses shall ensure only parents/guardians are allowed to take discharged children home. Only parents or legal guardians are allowed to take home discharged babies/newborns.
- v. All patients deemed fit for discharge shall be provided with a prescription and relevant information about their medication prior to discharge.
- vi. A diagnosis shall be made before a patient is discharged. Doctors have to complete and provide the Discharge Summary to the patient.
- vii. If the patient requires follow-up at the community level (General Practitioner, *Klinik Kesihatan*, Primary Care Medicine Clinic), a letter of referral shall be provided to the patient.
- viii. All discharged patients must settle their bills in accordance with the fees schedule.
- ix. Malaysians and Non-Malaysians who are unable to settle their bill due to financial reasons will be referred to the Medical Social Department/Revenue Unit/Hospital administration.
- x. Planned discharge should be done between 8 am till 9 pm.

4.7.2 Discharged Against Medical Advice (DAMA)

- i. All patients requesting to be discharged against medical advice can do so after obtaining adequate explanation and clarification from the MO/Specialist in charge.
- ii. The DAMA discharge form has to be completed by the MO in charge and signed by the patient/relatives/guardian and a witness. DAMA forms shall be retained at the Patient Record Department.
- iii. On discharge including DAMA discharge, patients shall be provided with relevant documents related to their admission, follow-up, medications and further management e.g. discharge notes, medical certificate, appointment card etc.

4.7.3 Absconded patient

- i. Patients are not allowed to leave the ward without permission. Those leaving the ward without permission shall be declared as 'absconded'.
- ii. If a patient is found to be missing from the ward/bed, all efforts shall be made to locate him/her within the vicinity of the Hospital. The ward staff shall notify the next of kin immediately.
- iii. The moment a patient is absconded or missing, a police report must be made immediately.
- iv. If the patient remains missing after 24 hours, the patient shall be removed from the census.

4.7.4 Dismissal of Patients from System

- i. The system will automatically discharge a patient from the clinic at 00:00 hours (at midnight). The exception to this is the ETD.
- ii. For in-patients, a similar function is made available for patients who have been discharged on the same day.

4.7.5 Discharge to Other Hospitals

- i. Discharges to other hospitals are allowed based on discussion and agreement between the referring and receiving Hospitals.

4.7.6 Discharge Dead

- i. The movement of a deceased patient shall be made to the mortuary and discharged from the mortuary.
- ii. Identification wristband of the dead body shall be removed in the mortuary upon body release.
- iii. All claimant of deceased must settle their bills in accordance with the fee schedule. Unsettled bills may cause delay in body release procedure.

4.7.7 Discharge Diagnosis

- i. Discharge diagnosis/diagnoses are made mandatory in the Electronic Medical Record (EMR) upon discharge from the Hospital or facility. These diagnoses shall be in free-text documentation. Diagnoses shall be classified into principal diagnosis, secondary diagnosis, comorbidities etc.

4.7.8 Cancellation of Discharge

- i. Cancellation of discharge before `midnight is permitted under specific conditions:
 - Clerical error e.g. the wrong person discharged or transcription error.
 - Unexpected change in patient condition.
 - The issue of logistics.
 - Cancellation shall be performed by authorised personnel in the ward. A request needs to be made to the authorised personnel (Information Technology Officer).

4.8 Death

4.8.1 Death at Hospital

- i. The attending doctor in the ward or the ETD shall carry out confirmation of death. Ward staff are responsible for informing next of kin immediately regarding the deceased. If unable to contact the family, police help must be sought to detect next of kin. Patients who died in the Hospital shall be transferred to the mortuary accompanied by Hospital personnel after one hour of confirmation of death.
- ii. On confirming death, the ward/department staff shall verify whether the deceased is a pledged organ donor. If yes, the staff shall notify the mortuary

and contact the next of kin. If the next of kin is not contactable, the police shall be notified.

- iii. The attending doctor, on confirming the death of a patient, shall register the death using the forms “*Perakuan Medical Officer Mengenai Sebab- Sebab Kematian JPN. LM9*” and “*Daftar Kematian/Permit Mengubur JPN.LM02 (Pin. 1/11)*”. House Officers shall not be allowed to sign the above documents.
- iv. Last office procedure including removal of all attached medical devices/instruments/items (except identification wristband and items deemed to be kept by the medical officer) must be done by respective ward staff. All wounds caused by iatrogenic/medical intervention must be closed/sealed completely and free of discharge.
- v. The body of the deceased must be tagged with a body tag bearing the identity of the deceased in addition to the identification wristband of the deceased. A white tag shall be used for non-police cases and a red tag for police cases.
- vi. All deaths in the Hospital shall be registered at the mortuary. Bodies shall be released to the next of kin or authorised person through the mortuary. All information on body release shall be documented.
- vii. In the case of a referred patient, the Hospital shall be responsible for body release to the next of kin or autopsy, if necessary.
- viii. The Hospital has the right to proceed with body disposal if no claim is made after 3 days for Muslims and 14 days for non-Muslims. Unclaimed bodies (non-police cases) shall be notified to the police immediately.
- ix. For unclaimed bodies of non-citizens, the respective embassies shall be notified of the death.
- x. Management and handling of infectious dead bodies shall be complied in accordance with the existing policies and practice guidelines to prevent cross- infection.^{[31][32][33]} Environmental Health Officer or Assistant Environmental Health Officer from the Environmental Health & Sanitation Unit shall be informed.
- xi. Subject to fulfilment of criteria and relevant procedures imposed by the Hospital, unclaimed bodies shall be handed to the Faculty of Medicine UniSZA for the purpose of education and research. For avoidance of doubt, reference may be made to *Garis Panduan Penyerahan Mayat-Mayat Tidak Dituntut di Hospital Kementerian Kesihatan Malaysia Kepada Medical Faculty Universiti Tempatan Bagi Maksud Pendidikan atau Penyelidikan Perubatan*.
- xii. Any patient who dies in the Hospital shall be transferred on a cadaver trolley to the mortuary by the mortuary attendants.

[31] *Garis panduan Pengimportan Atau Pengeksportan Mayat Atau Mana-Mana Bahagiannya Edisi Pertama Moh/K/Epi/ 2006.*

[32] *Policies and Procedures on Infection Control*, Ministry of Health Malaysia, Second Edition, 2010.

[33] *The Disinfection and Sterilization Policy and Practice 2002*, Ministry of Health.

4.8.2 Brought-in-Dead (BID)

- i. All BID cases brought by the police shall go directly to the mortuary.
- ii. BID cases brought by families/public shall be seen and registered in ETD and a police report shall be made by family members and ETD staff (without family members) the body will be transferred to the mortuary after discharge procedure has been settled.
- iii. The police shall decide the need for forensic post-mortem examination according to the cases.
- iv. For cases that require Crime Scene Investigation (CSI) as requested by the police, the Assistant Medical Officer (AMO) on duty shall inform the Forensic Pathologist on call immediately.
- v. The body shall only be released after all the relevant procedures, documentation is completed and clearance from the police is obtained.

4.8.3 Post Mortem

- i. For police cases, death should be reported to the police and a post mortem order will be issued by the Investigating Police Officer.
- ii. Forensic post mortem shall be performed by the Forensic Pathologist or MO from the Forensic Department depending on the complexity of the cases.
- iii. The clinical post-mortem examination may be requested by the specialist in charge of the deceased to study and identify the main reasons that led to the death after appropriate treatments and interventions that have been given fail to recover or save the individual. Clinical post-mortems are conducted in cases usually not under the investigation of police officers. Consent from the next of kin must be obtained and discussion with Anatomic Pathologist must be done before a post-mortem is performed.
- iv. Clinical post-mortem shall be performed by the Anatomic Pathologist with the presence of clinician in charge assisted by forensic staff.

4.9 Referral System

4.9.1 General

- i. Transfer of patients may occur routinely or as part of a regionalized plan to provide optimal care for patients at more appropriate and/or specialised facilities.
- ii. Referral of patients between Hospitals can occur from a lower to a higher level of care, higher to a lower level of care and also at the same level of care depending on the needs of the patients and/or the providers of care.
- iii. The Hospital shall develop pre-existing transfer arrangements between the facilities and pre-transfer communication between the appropriate persons to facilitate the efficient flow of the continuum of care to the patient.

4.9.2 Inter-departmental (In-patient) Referral

- i. Inter-departmental in-patient referrals shall be seen in the patient's respective ward (unless a special procedure is required).
- ii. Referrals shall be made with the approval of the attending Specialist/Consultant.
- iii. Referrals shall be duly seen by the MO/Specialist/Consultant.
- iv. Referrals shall be made online and followed by a courtesy call.
- v. Urgent referrals (decided by the receiving team) shall be seen within 6 hours or sooner, depending on the level of acuity.
- vi. Non-urgent referrals (decided by the receiving team) shall be seen within 24 hours of the referral.

4.9.3 Inter-departmental (Out-patient) Referral

- i. Referrals to the Specialist Clinics shall be done through the scheduling/appointment system.
- ii. Referrals shall be in a specified format and provide details of history, clinical examination, treatment and reasons for the referral.
- iii. Referrals to sub-specialty clinics shall be according to the criteria for referrals or admissions agreed upon by all departments.
- iv. Referrals to ETD from specialist clinics should be informed and accepted prior to transfer. Referrals should be done by MOs to ETD MOs and the respective primary team's MO on call.

4.9.4 Incoming External Referrals (Out-patients) Referral

- i. External referral includes referral from Government Health Facilities, General Practitioner, Private Healthcare Facilities.
- ii. Referral to ETD will be triaged to determine the stability of the patient, urgency of the referral and appropriateness to the department being referred.
- iii. All external referrals from MOH, General Practitioner (GP), *Pusat Kesihatan Universiti* (PKU), University Medical Centre (UMC), *Universiti Malaysia Terengganu* (UMT), Private Hospitals, etc. to ETD should be informed and accepted prior to transfer. Referrals should be done by MOs to ETD MOs and the respective primary team's MO on-call.
- iv. Referral to Specialist Clinic shall be assessed by a staff nurse/medical assistant at the clinic counters during office hours.
- v. Appointments for referrals shall be made at the main registration counter of the Specialist Clinic either through a telephone call, email, or by the patient presenting with relevant referral documents.
- vi. All urgent referrals for radiology procedures shall be through a primary team in the Hospital.
- vii. Scheduling of patients for investigation and consultation, where possible shall be made on the same day.

4.9.5 Department of Emergency and Trauma Referral

- i. For unstable patients, referrals shall be made between the MOs and if needed, between the ETD Specialist and the receiving department specialist.
- ii. For stable patients, referrals shall be made by MOs to MOs
- iii. All ETD referrals must be seen by the respective MO/Specialists. No cases should be refused or discharged via phone calls. All cases referred must have written documentation by the primary team.
- iv. Patients at the resuscitation zone shall be seen immediately by the referred MO/Specialist within 15 minutes of referral.
- v. Patients at the yellow zone shall be seen by the referred MOs/Specialist within 45 minutes of referral.
- vi. Patients at the green zone shall be seen by the referred MOs/Specialist within 60 minutes.
- vii. If a patient is still not seen by the referred team within the stipulated time, emergency physician on duty has the right to directly admit the patient to the respective ward.
- viii. All cases that have been seen by the referred team and deemed to be referred to the other specialty(es) should be done so by the referred team.
- ix. In case of any dispute in the disposition of patients towards any specialty, the ultimate decision will be made by the emergency physician on duty/Head of Department (HOD) of ETD.

4.9.6 Intra-Facility Transfer

- i. All unstable patients shall be accompanied by trained personnel including the primary team's MO during transfer.
- ii. All critically ill patients and those requiring assisted ventilation from ETD may be admitted directly to the critical care ward after being reviewed by the primary team and after consultation with the department's specialist and anaesthetist-in-charge of the intensive care ward.
- iii. Both referring and admitting Specialist/Consultant shall agree on the transfer decision.
- iv. A patient shall only be transferred to another ward upon approval by the specialist.
- v. All transfers including bed swapping shall be documented in the Total Hospital Information System (THIS).
- vi. Patients and next of kin are to be informed of the transfers.
- vii. Patient's/family's request for transfer to a different bed category shall be accommodated upon the availability of bed and affordability once the patient is in ward.

4.9.7 Inter-Facility Transfer

- i. Patient transfer is a doctor-to-doctor referral.
- ii. The decision to transfer a patient for higher level care shall be made by the primary team, upon consultation with the specialist concerned.
- iii. The referring team must contact the relevant MO/Specialist at the receiving Hospital to discuss the necessity of the transfer of a patient. The receiving MO/ Specialist must agree to accept the patient before the transfer takes place.
- iv. The patient's next of kin shall be informed about the process of transfer. In emergencies when a patient is unable to agree to transfer, and the next of kin is not contactable, the police officer shall be informed to help in contacting them. The decision for transfer rests on the MO/ Specialist in charge of the patient and the consent of the relatives is not always required.
- v. All patients shall be stabilised and deemed stable before transfer.
- vi. Patients requiring transfer to other hospitals shall be accompanied by the primary team's personnel. The staff accompanying referred cases shall be decided by the MO or specialist in charge, after consultation with the receiving Hospital.
- vii. All critical patients shall be accompanied by paramedics trained in resuscitation and led by a MO of the referring team; Accompanying staff for other cases shall be decided by the MO/Specialist in charge based upon the clinical condition of the patients. Continuous monitoring of patients shall be done and recorded accordingly.
- viii. Documents pertaining to a patient's condition shall be made available to facilitate the transfer. This includes a referral letter with a detailed history of the patient and the reason for the referral. All related radiological images and other investigation results (e.g. blood results) shall be included.
- ix. A patient may be referred to the ETD or directly to the appropriate ward/care unit including intensive care. The accompanying team must be from the primary team and shall have clear instructions as to their exact destination (e.g. which ward to go to) before arrival at the receiving Hospital to avoid unnecessary delay.
- x. The accompanying team shall not leave the patient until the receiving team has formally taken over the care of the patient for up to 2 hours.
- xi. If the patient's clinical condition deteriorates during the transfer and resuscitation is required, the ambulance may divert to the nearest health facility or be directed immediately to the ETD of the receiving Hospital.
- xii. If death occurs during the transfer, it shall be certified by the accompanying MO and the body shall be brought back to the referring Hospital.
- xiii. Patients requesting self-transfer to other centres shall be informed of the risk by the attending doctor/specialist. The Hospital is not responsible for making transfer arrangements for self-transfer patients.

- xiv. Patients on self-request transfer must sign the DAMA form and be given a discharge note before leaving the premises.

4.10 Medical Records and Documentation

4.10.1 Electronic Medical Record (EMR)

- i. EMR shall be created upon registration of patients in the Hospital.
- ii. Clinical management of all patients inclusive of data and images shall be entered in the EMR of THIS by healthcare providers.
- iii. Documentation of clinical care/data entry shall be maintained by Hospital personnel attending to the patient and each entry is captured in a real-time manner.
- iv. Data entry is to be keyed-in by the healthcare provider or flowed in automatically from other systems/applications.
- v. All supporting systems i.e. Laboratory Information System (LIS), Radiology Information System (RIS), Picture Archiving Communication System (PACS) etc. shall be integrated into the main THIS.
- vi. EMR is accessible in the system for those with access rights until 24 hours after the visit (Out-patient) and 72 hours after discharge (In-patient).
- vii. Medical Officer and Specialist will be given access to view the patient's document in EMR when needed.

4.10.2 Summaries in Electronic Medical Record

There shall be two types of summaries depending on the services rendered which are Encounter Summary and Discharge Summary. Accurate International Classification of Diseases (ICD) Coding shall be used for diagnosis.

A. Encounter Summary

- i. Encounter Summary is defined as a summary made at the end of every encounter by the Health Care Provider in the out-patient setting.
- ii. Encounter Summary shall be prepared once the patient has completed seeing the Health Care Provider such as in ETD, Day Care Services, Specialist Clinics, Rehabilitative Clinics, Dietetic Clinic, etc.
- iii. Encounter Summary is auto-generated once the healthcare provider discharges the patient. The defined content shall be auto-populated in the Encounter Summary. Before submission of the Encounter Summary, the healthcare provider shall confirm the content of the Encounter Summary and be able to edit wherever necessary, except for the demography information.
- iv. Encounter Summary shall be completed within 24 hours.
- v. A Medical Officer shall verify the Encounter Summary if it is prepared by the House Officer (HO).

B. Discharge Summary

- i. A Discharge Summary is defined as a summary of the patient's illness and management rendered to him/her during the episode of stay in the Hospital.
- ii. A Discharge summary shall be prepared by the doctor once a patient is discharged from the ward regardless of the type of discharge.
- iii. Types of discharges are:
 - Discharged home
 - Death
 - Absconded
 - Discharged against medical advice (DAMA)
 - Transferred out
- iv. A Medical Officer shall verify the Discharge Summary if it is prepared by the House Officer.
- v. The doctor from the discharging discipline shall prepare the final Discharge Summary.
- vi. The doctor shall ensure completeness of the Discharge Summary by entering (key-in) Principal Diagnosis, Secondary Diagnosis, Co-morbidities, Complications etc.
- vii. The Discharge Summary of a deceased patient shall be prepared using a similar format of the Discharge Summary; however, information regarding death is made available.
- viii. Discharge Summary can be accessed following the Hospital User Access Control Policy (UACP).

4.10.3 Access and Sharing of Electronic Medical Records (EMR) in THIS.

- i. In digital technology, there are rules pertaining to access and retrieval of records. This can be referred to the Hospital User Access Control Policy (UACP) and existing laws and regulation.^[34]
- ii. Data may be accessed by Physicians, Medical Record Department, Information Technology Department.
- iii. Physicians can fully access EMR for 24 hours for out-patient.
- iv. Physicians can fully access EMR for 72 hours for in-patient after patient discharge.
- v. After 24 hours' treatment for out-patient and 72 hours after the patient has been discharged, physicians are not allowed full access to the system but will be allowed to view only the documents/loose notes of patients.

^[34] Personal Data and Protection Act 2010 [Act 709] Laws of Malaysia.

4.10.4 Principles to Sharing of Data

- i. Confidentiality of patient data is of most importance and shall be strictly observed at all times.
- ii. Data sharing is the responsibility of the healthcare provider. He/she must weigh the benefits and risks of sharing data of the patient, the health care provider and the organisation (stakeholder/institution/Hospital).
- iii. Patient's data is not allowed to be shared, printed or given to other people.

4.10.5 General Conditions for Access of Electronic Medical Record in a Facility

- i. In general, EMR can only be accessed in the following conditions:
 - When the patient is admitted to the hospital.
 - When the patient is referred to the hospital.
 - In certain cases, under the jurisdiction of the Hospital director, e.g. Updating the national registry or quality initiative activities.
 - For clinical coding diagnosis by a doctor, check and by medical record officer or authorised personnel at Medical Record Department.
 - If the Medical Record Officer (MRO) finds out the diagnosis by the doctor is not correct, MRO will notify the doctor with the suggestion code through the THIS system.
 - Doctors need to do the correction for diagnosis on THIS.
 - Case to case basis, upon need-to-know for clinical or research purposes, only for registered medical personnel or authorised personnel and can be audited and monitored by the Information Technology Department.
- ii. Printing of any part of the EMR shall not be allowed except for referral purposes. Printing of the EMR shall be allowed if required by the law. Printing of any part of the EMR shall be done in the Patient Information Office. However, referral letters/consent forms/specified identified documents can be printed in the ward or clinic.
- iii. The Hospital Director is responsible for determining the access of VIP and medico-legal cases.
- iv. The Custodian to the EMR shall be the Medical Record Department. However, access control of the EMR is under the purview of the Information Technology Department.
- v. For an inactive user of more than six (6) months, access shall be denied by the Department of Information Technology.
- vi. Access for Continuity of Patient Care.
 - EMR can be accessed by officers in the attending department.
 - Clinical Support Service Officers e.g. Diagnostic Imaging, Pathology,

Rehabilitation, Physiotherapy, Dietetics and Pharmacy, can access the demographic and clinical data of patients who receive consultation in their disciplines.

- Access to EMR shall be deactivated after 72 hours (working day) of patient discharge from in-patient services and after 24 hours (working day) of out-patient services.
- Access will be given to physicians just for view only.

4.10.6 Purpose of Access

i. Access for Continuity of Patient Care.

- EMR can be accessed by officers in the attending department.
- Clinical Support Service Officers e.g. Diagnostic Imaging, Pathology, Rehabilitation, Physiotherapy, Dietetics and Pharmacy, can access the demographic and clinical data of patients who receive consultation in their disciplines.
- Access to EMR shall be deactivated after 72 hours (working day) of patient discharge from in-patient services and after 24 hours (working day) of out-patient services.

ii. Access for Research and Study Purposes

- Access to data in HPUniSZA facilities is allowed with written approval from the Hospital Director. The Hospital Director can appoint an officer for access rights approval.
- Clinical related research involving patient care shall obtain approval from the Research & Ethics Committee, and audit-related research shall obtain approval from the Hospital Audit Committee.

iii. Internal Staff

- For internal staff, data can be extracted through the Reporting Module in THIS.
- EMR access shall be made under the name of the applicant.
- Only specific approved EMR can be accessed by the requestor.
- The validity period of access rights is given upon request by the applicant with approval from the Hospital Director.

iv. External Staff

- External staff are defined as those who are not working at the facility. External staff must include UniSZA staff as a member in the research or study project.
- External staff need written approval from the Hospital Director.
- Temporary access shall be given to the requestor by the system administrator.

- Access duration will depend on the Hospital Director's approval in the facility.
- Access rights should be automatically disabled once the research ends.
- Students may be given access with approval by the Hospital Director but are restricted to view only.
- External staff need to comply with the research guidelines endorsed by the Hospital Medical Research and Ethics Committee.
- Information sharing between the Ministry of Higher Education (MoHE) and other governmental agencies is allowed with mutual agreement.
- The respective agencies or institutions shall have a Memorandum of Understanding (MOU)/Memorandum of Association (MOA)/Non-Disclosure Agreement (NDA) before data sharing.

4.10.7 Amendment of Electronic Medical Record

- i. An amendment is defined as any changes made to the original record e.g. addition, deletion and substitution.
- ii. In EMR, all amendments shall be made through soft delete (strikethrough) and a new entry shall be keyed-in in italics next to it. The system shall show the date and time of changes made.
- iii. The system shall not allow cut and paste functions for amendment purposes.
- iv. Amendments can be performed within 72 hours of discharge.
- v. Any amendment deemed necessary after 72 hours of discharge needs approval from the Medical Record Department with permission from the HOD. A grace period of 14 days is permitted for amendments to be made.

4.10.8 Addendum of Electronic Medical Record

- i. An addendum is defined as any form of data or image added to the EMR.
- ii. Reasons for addendum are inclusive of but not limited to an update of investigation results, the addition of Medical Certificate (MC) and referral letter.
- iii. There is no time limit for an addendum to be done.
- iv. There is no limit in terms of the quantity of addendum to be made.
- v. The system should capture and display the person who made the addendum, and the date and time it was performed.
- vi. The system shall also capture and display the reason for an addendum.
- vii. There shall be no amendment allowed for Encounter Summary or Discharge Summary. However, an addendum is allowed to the Encounter Summary or Discharge Summary which provides data to be added to the primary document before the Encounter Summary or Discharge Summary.

4.10.9 Total Hospital Information System

- i. THIS shall be an enabler of EMR that allows better sharing and record keeping.
- ii. Staff who have been granted access to patient medical records shall follow the policies and laws that govern the use and disclosure of patient medical records. The level of access in THIS is given following rules and User Access Control Policy (UACP).
- iii. All staff can be accountable for any medico-legal matter from incomplete documentation in THIS. The Hospital Director, Information Technology Department and the Medical Record Department shall actively monitor the accuracy and relevance of information in THIS.
- iv. Any external request to access the patient record shall be made in writing/email to the Hospital Director to ensure the confidentiality of the patient's medical record.
- v. Students or private practitioners shall not be given direct access to THIS. However, restricted access may be given with the approval of the Hospital Director according to ICT Policy.
- vi. Recent advancement of technology allows the use of BYOD (Bring Your Own Device). Usage shall abide by the policy of BYOD.

4.10.10 Management of Medical Records and Report

- A. Medical Record
 - i. Every patient receiving care in the Hospital shall have an individual medical record in the EMR.
 - ii. The care given and procedures done on a patient must be documented in the patient's medical record. The attending doctor shall be responsible for proper documentation and legibility of the notes in the record. All health care providers shall enter patient progress into the system based on their Login (Identification Data) ID.
 - iii. Every patient must have either a provisional or final diagnosis recorded on discharge or upon death.
 - iv. The diagnosis and cause of death for all patients shall follow the ICD code.
 - v. Referral letters and other hard copy documents which are not in EMR and related to the patient's care shall be digitised and hard copies to be kept in the patient's case notes.
 - vi. All referred radiological images from external Hospitals should be digitised and the original copies returned to the referring Hospitals or the patient.
 - vii. No hard copy of medical records and documents shall be allowed to be brought out of the Medical Record Department without prior permission from authorised personnel. The medical reports or documents are not allowed to be taken out of the Hospital.

- viii. All records and information of patients are confidential. The patients' medical records shall belong to the Hospital. Release of patient information to other parties, except by court order requires prior consent from the patient or relevant authorities.
- ix. Management of medical records shall be under the responsibility of the Medical Record Department. The records shall be managed to ensure safety, confidentiality and fast retrieval.
- x. All other personnel involved in the handling of medical records shall also be responsible for maintaining the confidentiality and safety of the records.
- xi. A Medical Record Committee (MRC) shall coordinate all issues pertaining to medical record services.
- xii. The management of patient medical records shall comply with existing guidelines.^[35]
- xiii. Refer to the circular by MOH and *Jadual Pelupusan Rekod Perubatan 2016* regarding medical record disposal.^[36]
- xiv. The disposition of hard copies of medical records must comply with existing law and regulation.

B. Medical Report

- i. A medical report shall be prepared upon receiving a written request from the patient or authorised person. The medical report shall be prepared with reference to the content in the patient's medical record.
- ii. The medical report shall be prepared by a MO or Specialist in the respective discipline involved in the care. The completed medical report applications are to be processed and provided within 28 working days from the date of application.
- iii. A medical report that has been officially released shall not be altered or tampered with. Any party i.e. patient, lawyers or insurance company may request verification when there is suspicion of tampering of the medical report. The Hospital shall verify that it is 'similar' or 'not similar' to the original report released by the Hospital.
- iv. Medical reports of medico-legal or potential medico-legal cases shall be prepared by the doctor or specialist managing the case and checked by the Head of Department and verified by the Hospital Director before release.
- v. The medical report shall be charged in accordance with the Hospital fees schedule, Fees Act 1982 and research studies from teaching hospitals and other hospitals.^[37]

^[35] Pekeliling Ketua Pengarah Kesihatan Bil. 17/2010: Garispanduan Pengendalian Dan Pengurusan Rekod Perubatan Pesakit bagi Hospital-Hospital dan Institusi Perubatan.

^[36] Pekeliling Ketua Pengarah Kesihatan Bil. 13/2016 (Pelupusan Rekod Perubatan).

^[37] Fees Act 1982- Fees (Medical) (Amendment) Order, 2016.

- vi. Existing guidelines shall be complied with in the preparation of the Medical Report.^[38]
- C. Medical Statistics
 - i. Statistics and reports shall be generated by the THIS system, as specified by the Hospital Management or the Medical Record Committee (MRC) of the Hospital.
 - ii. The respective department/unit shall submit data to the Medical Record Department within the specified time.
 - iii. Request for medical data and statistics of the Hospital shall be done through the Medical Record Department and release is subject to the Hospital Director's approval.
- D. Medical Board
 - i. A Medical Board may be established under eight (8) circumstances, according to guidelines from MOH.^[39]
 - ii. A Medical Board application must be made through the State Health Office or the Hospital Director's Office.
 - iii. Application for other circumstances than those in the guideline shall be sent in writing according to the reasons.
 - iv. The application for the Medical Board shall be charged following the fees schedule.^[37]

4.11 Procedures and Surgery

- i. Each procedure or surgery is planned and documented well in the patient's case notes. All elective surgical cases requiring anaesthesia will be reviewed in the anaesthesia clinic and shall be referred to the relevant department for further assessment or optimization if required.
- ii. Referral to Anaesthesia Team shall be made for emergency surgery if there is any concern of fitness for surgery.
- iii. All consent must be taken before any procedure or surgery.
- iv. Upon arrival at the OT, the OT nurse must verify with the relative/patient the following based on a checklist:
 - Patient's details by verbal questioning and Hospital identification tag
 - A consent signed by relevant parties
 - Type of operation
 - Site of operation (if relevant)

^[38] *Pekeliling Ketua Pengarah Kesihatan Bil. 16/2010: Garispanduan Penyediaan Laporan Perubatan di Hospital-Hospital dan Institusi Perubatan.*

^[39] *Buku Garispanduan Penubuhan Lembaga Perubatan Di Jabatan Kesihatan Negeri, Institusi Perubatan & Hospital-Hospital Kementerian Kesihatan Malaysia, June 2010, m/s 2-3.*

^[37] Fees Act 1982- Fees (Medical) (Amendment) Order, 2016.

- v. The surgery performed shall be recorded using a prepared format and attached to the patient's case notes. Documentation should include the postoperative diagnosis, the description of the surgical procedure, the findings and any surgical specimen sent and the name of the surgical and anaesthetic team members (attending doctors and nurses). The patient's postoperative care plan shall also be documented.
- vi. The safety of the patient undergoing surgery shall adhere to the Safe Surgery Saves Lives (SSSL) Initiative.^[40]

4.12 Drugs and Medication

4.12.1 Usage

- i. HPUniSZA Drug Formulary shall be maintained and used as a guide for drug prescription. Any omission and exclusion of drugs shall be discussed and decided in *Jawatankuasa Ubat dan Terapeutik* HPUniSZA.
- ii. Prescription and supply of drugs not listed in the HPUniSZA Drug Formulary shall require approval by the Hospital Director.
- iii. The respective Head of Department shall be responsible for justifications of drug usage and cost implication. Requests for approval shall be made using *Borang Permohonan Ubat Bukan Standard/Standard Terkawal* and submitted to the Pharmacy Department.
- iv. All operational processes under the Pharmacy Department shall follow existing laws and regulations.^{[23][41]}

4.12.2 Prescription

- i. Authorised doctors shall prescribe drugs only to registered patients.
- ii. A prescription must be done through EMR. A manual prescription will not be entertained unless there are technical issues except ETD.
- iii. Prescription (more than 1 month) shall be filled in at specified intervals.
 - a. Patients with new prescriptions shall collect their medicines within one week from the prescription date or before the prescription expires.
 - b. Patients with Balance Medication Sheet (BMS) shall collect their medicines within one week from the collection date or before the prescription expires.

4.12.3 Dispensing

- i. Drugs shall be dispensed at the specified pharmacy counter for one-month duration and subject to the availability of drugs.
- ii. Drug counselling shall be provided to individual patients based on needs.
- iii. All dispensing processes shall be done by trained pharmacy personnel.

^[40] Safe Surgery Saves Lives (SSSL), 2nd Edition 2018.

^[23] Dangerous Drugs Act 1952 [Act 234] Laws of Malaysia.

^[41] Poison Act 1952, Laws of Malaysia.

- iv. Urgent needs after office hours for in-patients shall be dealt by the pharmacy personnel on call.
- v. Value-added services for drug collection such as Medicine Delivery, Leave, “N” Pick, Book “pick” and Drive-Thru shall be established to improve the efficiency of drug dispensing.
- vi. Patients who brought their medication will have this medication served to them by the staff nurse.

4.12.4 Monitoring

- i. Usage of drugs, prescriptions and inappropriate drug reaction shall be monitored (medication error reporting & Adverse Drug Reaction) by the Pharmacy Department. The Department of Pharmacy will coordinate, monitor and manage all issues relating to drugs and drug usage.
- ii. Monitoring in terms of current stock level including slow-moving and near expiring medication shall be done regularly by the Pharmacy Department under the Main Store to ensure optimal usage of drugs.

4.13 Sterilisation and Disinfection

4.13.1 Sterilisation and Disinfection Services

- i. The CSSD shall be overall responsible for the sterilisation and disinfection services in the Hospital.
- ii. Sterilisation and disinfection of equipment and surgical items shall be carried out using the appropriate and accepted techniques or methods.
- iii. Staff involved in the sterilisation process shall follow the standard procedures to ensure the sterility of the product.
- iv. Staff shall wear proper attire for safety protection against infection and other hazards.
- v. The Department shall ensure that equipment is in good condition and develop a plan for the restoration and replacement of non-functioning equipment by the user.
- vi. Sterilisation of delicate equipment shall be carried out by trained staff using appropriate techniques. Soft dressing shall be pre-packed and sterilised centrally.
- vii. For a high-risk patient, such as a known case of HIV/Acquired Immunodeficiency Syndrome (AIDS) and Hepatitis B Virus (HBV)/Hepatitis C Virus (HCV) disposable sets shall be used when available.
- viii. Sterile materials shall be collected by all wards, clinics and OT at CSSD by themselves.
- ix. Utilisation of sterile disposable items or equipment is preferred wherever permitted by the availability of the resource.
- x. Sterilisation of instruments from other institutions requires authorisation from the Hospital Director.

- xi. Loaned equipment from the vendor shall be sent to CSSD 24 hours before the procedure.
- xii. Loaned equipment from CSSD shall be returned to CSSD 24 hours after the procedure.

4.13.2 Sterile Supplies

- i. The CSSD shall receive and return sterile medical instruments and sterile linen from the wards and departments on a regular basis.
- ii. Used CSSD packs from the departments shall be kept in a clean transport trolley after the initial rinse and sent to CSSD for cleaning on a regular basis.
- iii. Sterilisation of instruments and materials shall be at the CSSD, except for:
 - Flash sterilisation of dropped equipment at the OT.
 - Sterilisation of milk bottles in the milk kitchen
 - Sterilisation of pharmaceuticals at the Pharmacy Department
 - Sterilisation of laboratory media, glassware and infectious specimens at the Clinical Diagnostic Laboratories.
- iv. Disinfection of certain medical equipment and materials will be managed in the Hemodynamics Room or Day Care Unit and shall be the responsibility of the respective department.

4.14 Infection Control

- i. The Hospital Infection and Antibiotic Control Committee (HIACC) shall be established and consist of an Infectious Disease Consultant or Clinical Microbiologist, assisted by an Infection Control Doctor and an Infection Control Nurse. This committee shall be chaired by either the Clinical Microbiologist or Infectious Disease Consultant.
- ii. HIACC shall coordinate all activities related to infection control. Issues pertaining to hospital infection shall be presented to the Committee for further action.
- iii. An Infection Control Coordinator shall be appointed by the Hospital Director. The coordinator, together with the liaison officer (link nurse/staff) from each area/ward, shall form the Infection Control Team.
- iv. The team shall monitor the implementation of infection control procedures, carry out surveillance activities, monitor antibiotic resistance patterns, hand hygiene compliance and conduct training of Hospital staff.
- v. The team shall coordinate surveillance activities for the hospital and collect relevant information including point prevalence studies on HCAI, antibiotic audits, hand hygiene compliance etc.
- vi. Infectious patients shall be placed and nursed in single rooms or negative pressure rooms wherever possible. The use of multi-bedded rooms for the same type of infection or microorganisms is acceptable.
- vii. The Infection Prevention and Control Team in collaboration with the relevant clinical

team, should be proactive in assessing the high-risk patients and risk of transmission of Multidrug-Resistant Organisms (MDROs) especially in the high-risk areas or units (e.g., Centralised Intensive Care Complex (CIC), Neonatal Intensive Care Unit (NICU).

- viii. Staff shall be instructed to adhere to barrier nursing and standard precaution guidelines at all times. This includes frequent hand washing and the use of personal protective equipment by those having direct contact with an infectious patient following hand hygiene practice. Standard precautions should be strictly adhered to at all times, irrespective of patient placement.
- ix. Personnel with occupational exposure to HIV/AIDS, HBV and HCV infections should follow recommendations for Post Exposure Prophylaxis (PEP) to prevent possible transmission.
- x. The staff who transports and receives the infectious patient should at all times adhere to Standard Precaution practice and appropriate Personal Protective Equipment (PPE) should be worn according to the risk of transmission as outlined in section of the Fundamental Principles of Infection Prevention, when in contact with the patient (before, during and after transfer).
- xi. The Infection Prevention and Control Team must be notified prior to the transfer taking place so that they can communicate transfer details with the receiving Infection Prevention and Control Team (where applicable) in order for appropriate infection prevention and control measures to be put in place to prevent the potential spread of infection to other patients, staff and visitors.
- xii. All instruments and linen used by infectious patients shall be placed in special bags (without washing or soaking). Staff shall place contaminated linen directly into a laundry bag in the isolation room/area with minimal manipulation to avoid contamination of air, surfaces and persons.
- xiii. All single-use medical equipment should preferably not be re-used. Healthcare facilities should ensure that all reusable medical equipment (e.g. blood glucose metres and other point-of-care devices, surgical instruments, and endoscopes) is cleaned and reprocessed appropriately prior to use on another patient. Reusable medical equipment must be cleaned and reprocessed (disinfection or sterilisation) and maintained according to the manufacturer's instructions and MOH protocol. Healthcare Workers (HCWs) must wear appropriate PPE when handling and reprocessing contaminated patient equipment.
- xiv. All clinical waste from infectious patients shall be double-bagged in yellow plastic bags for disposal by incineration. Management of the clinical waste shall be as stipulated in the contract. Refer to practice guidelines.^{[42][43]} Poor management of health care waste potentially exposes health care workers, waste handlers, patients and the community at large to infection, toxic effects and injuries, and risks polluting the environment.

4.15 Health Education

- i. The Hospital shall provide effective health/patient education services in support of in-patient and out-patient care in the Hospital.

[42] SPKPK Bil.6/1994 Garispanduan Untuk Membuang Alat-alat Suntik, Alat-alat Tajam dan Jarum Yang Telah Digunakan Di Hospital, Klinik Dan Pusat Kesihatan Di Dalam Sektor Kerajaan Dan Swasta, 13 September 1994

[43] SPKPK Bil.2/1990 Guidelines On Control of Hospital Acquired Infections, 7 February 1990

- ii. The Health Education Department/Unit headed by Health Education Officer, in collaboration with the Department of Public Health shall plan, coordinate, implement, monitor and evaluate all activities related to health/patient education programs in line with current HPUiSZA policies.
- iii. The Department of Public Health, in collaboration with the Health Education Department/Unit, shall plan, coordinate, implement, monitor, and evaluate all activities related to health promotional activities to the staff and Hospital clients in line with current HPUiSZA policies.

4.16 Organ Donation

- i. The Hospital Director shall establish an Organ Donation Committee to coordinate the process of organ donation and procurement.
- ii. All potential cases for cadaveric donations shall be made known to the committee.
- iii. Policies and procedures shall be made available to guide the procurement, donation process and transplantation of organs and tissues. They are consistent with the relevant laws and regulations and respect the community values, spiritual beliefs and religions.^{[44][45]}

4.17 Ethics and Law

The Hospital shall abide by the laws of the country, policies and guidelines of the University, the Ministry of Health, medical ethics and relevant policies and guidelines of other Ministries. Legislations, regulations, policies and guidelines may be amended by the relevant authorities as and when necessary.

^[44] National Organ, Tissue and Cell Transplantation Policy, 2007, MOH

^[45] SPKPK Bil 4/2008 Pekeliling KPK BIL 4-2008 - Perkhidmatan Perolehan Organ Dan Tisu Kadaverik

Chapter 5:

Medical Facilities

5.1 Specialist Clinics

- i. Specialist Clinics shall operate from 8.00 am to 1.00 pm, 2.00 pm to 4.45 pm on Sunday – Wednesday and 8.00 am to 1.00pm, 2.00pm to 4.30pm on Thursday.
- ii. All specialist clinics shall provide specialist out-patient care under the supervision of the specialist in charge and shall remain operational during office hours.
- iii. Whenever necessary, Consultation & Examination rooms shall be shared between the various departments/units. Throughout the consultation, the patient's privacy must be maintained based on a predetermined clinic schedule. If a patient requires private consultation, the patient shall inform the officer in advance so that a setting that respects the privacy of the patient can be arranged.
- iv. Access to specialist out-patient clinics is through written/email/applications/web referrals and appointments only. The respective department shall have appropriate processes in place to manage specialist out-patient services. Appointments for external referrals shall be obtained at the Specialist Clinics Registration Counter.
- v. A staggered appointment system shall be used to schedule patient appointments. Every effort shall be taken to ensure appointments take place at, or as close as possible to the scheduled appointment time. In the event the patient arrives in advance or later than 30 minutes of the scheduled appointment time, the patient may be accommodated if possible, or maybe offered another appointment at the discretion of the treating clinician.
- vi. Referral letters shall be sent to the Medical Records Office for documentation. (Need or not/EMR).
- vii. Specimens collection shall be at designated rooms and sent to the main laboratory.
- viii. Patients with prescriptions shall collect their medication at the Out-patient Pharmacy.
- ix. Allocation of appointments for patients accessing specialist out-patient clinics is based on prioritisation, clinical urgency and available resources.
- x. Rescheduling, cancellation and deferral of appointments shall follow the policy of the Medical Record Department.
- xi. All patients accessing specialist clinic services must provide the relevant documents to facilitate appointment scheduling and registration, such as referral letter, appointment card, guarantee letter etc.
- xii. Payment-related services shall be charged according to the Hospital Fees and receipt issued.
- xiii. All clinics shall display their client charter, which must be consistent with the daily services rendered. All clinics must ensure that the duration to obtain an appointment for all new patients is within a reasonable period. This must be monitored regularly by the clinic for continuous improvement
- xiv. Details of the clinic consultation shall be documented in the EMR. At the end of the clinic session, the patient may be either discharged, given another clinic appointment,

referred elsewhere or admitted to Day Care/Wards for investigations/procedures/treatment.

5.2 Department of Emergency and Trauma

The Hospital shall provide pre-Hospital and Hospital emergency services on a 24-hour basis. The Department shall be responsible for the provision of emergency care to patients to save lives, preserve body functions and prevent complications. The services shall be under the responsibility of the Head of ETD.

- i. The department shall have designated areas or zones for the management of patients according to the severity of illness.
- ii. All patients shall be triaged according to the Malaysian Triage Category (MTC); resuscitation zone for the critically ill, yellow zone for the semi-critical and green zone for the non-critical zone. Critically ill patients shall be seen immediately by the attending doctor.
- iii. Waiting time for the non-critical zones shall be displayed at the ETD. Locum services may be provided upon approval by the Hospital Director.
- iv. An observational medicine area shall be provided for the observation of a selected group of patients presenting to the ETD and subjected to a predefined care plan. The period of observation shall not exceed 4 hours unless clinically appropriate. The pre-admission management plan shall be carried out by the managing doctor.
- v. During a disaster, the Department shall play a lead role in the initial management and treatment of the victims on-site and inside the Hospital.
- vi. Standby medical cover shall be coordinated by the Department and provided on request subject to the availability of staff and the policy and procedures of HPUniSZA.

5.3 Daycare

The Daycare unit shall be utilised by relevant clinical disciplines for medical treatment, endoscopy and elective surgeries. A record of the procedures performed at Daycare will be maintained.

- i. Daycare services shall run from Sunday to Wednesday (working days) from 7.00 am to 10.00 pm.
- ii. All patients scheduled for Daycare services shall be given written information regarding registration and admission. Consent for surgery or other special procedures shall be obtained in a separate consent form by the doctor who scheduled the case for a daycare procedure.
- iii. Confirmation of patients undergoing daycare procedures must be made at least 24 hours before the procedure by the daycare staff.
- iv. Patients undergoing daycare procedures must be pre-selected, assessed and categorised as low-risk patients. These patients shall be registered and discharged within the same day after the surgery/procedures.
- v. Patients shall be certified fit by a MO before discharge; if they are deemed unfit, they

shall be admitted for further in-patient management.

- vi. Daycare procedures and management shall adhere to the Ministry of Health Malaysia guidelines on daycare service.
- vii. On registration at the Daycare, fees shall be collected before the procedure is performed. If there shall be any change in the procedure, the difference in the charges shall be reimbursed to or paid by the patient.
- viii. Billing for these patients shall be in accordance with the Hospital Fees.

5.4 Operation Theatre (OT)

- i. The Hospital management, guided by the surgical directorate, shall be responsible for providing OT facilities with adequate personnel and equipment to cater for elective and emergency procedures involving general, regional and local anaesthesia.
- ii. All elective surgeries shall be carried out between 8.00 am to 4.45 pm on Sunday to Wednesday according to the schedule based on the allocated OT days.
- iii. Provision of an extended OT up to 6.00 pm is allowed based the allocated schedule, subject to the availability of OT personnel and discretion of anaesthetist in charge.
- iv. Emergency OT shall be operational 24 hours a day. For category 1 and life-threatening obstetric cases, additional emergency OT shall be opened at the discretion of the anaesthesiologist in charge.
- v. The Anaesthetic MO or Specialist shall assess all patients undergoing elective and emergency surgery.
- vi. Patient safety is of utmost importance. To ensure that, all procedures carried out in the OT shall comply with Malaysian medical regulating bodies guidelines and policies including the Safe Surgery Save Lives (SSSL) initiative.

5.5 Intensive Care

5.5.1 Admission

- i. Admission to Centralised Intensive Care Complex (CIC) is based on the request of the primary team or the attending doctor. The decision to admit is based on the Intensivist in charge of the unit if the patient meets the requirement for intensive care as well as the availability of intensive care facilities and personnel.
- ii. Priority for admission shall be based on the urgency of the patient's need for intensive care. Unscheduled, emergency admission shall take precedence over scheduled elective surgical admission. Triaging of admissions to the unit shall be done by the Intensivist in charge of the unit in CIC.

5.5.2 Discharge from Intensive Care

- i. The decision to discharge from the unit in CIC to another ward/facility is based on the Intensivist in charge of the unit in CIC when the reason for admission has

been resolved. A Discharge Summary must be completed and attached to the patient's case notes.

- ii. When continuing intensive care for a patient is deemed medically futile, end-of-life care shall be considered. This decision shall be discussed with the patient's family and with other team members as appropriate. Referrals to Organs Procurement Team shall be initiated whenever possible.
- iii. The Specialist of the primary department/unit shall discuss with the Intensivist when relatives/next of kin request termination of treatment and DAMA for ill ventilated patients. Adequate explanation and the risks shall be made known prior to approval for discharge.
- iv. If the relatives of the patient requested Discharge Against Medical Advice (DAMA), extubation of the patient and discontinuation of medical therapy shall be carried out either in the CIC/ambulance and shall not be accompanied home by CIC nurse or paramedic. Intensivists or medical officers shall discuss and provide adequate explanations including the related risks and cost of private ambulance transfer shall be borne by the relatives.
- v. In cases where relatives/next of kin request for ill-ventilated patients to be transferred to other medical facilities, the Intensivist shall discuss and provide adequate explanation including the risks involved prior to approval. Pre-transfer communication between the Specialists of the referring and receiving unit/facility shall be documented.
- vi. For a referral to a private facility on patient's request, the arrangement for the transport and care during the transfer shall be the responsibility of the relative/next of kin and may be facilitated by the Intensive Care personnel.

5.5.3 Management

- i. Patients admitted to each unit of the Centralised Intensive Care Complex (Intensive Care Unit (ICU), High Dependency Ward (HDW), Post-Anaesthesia Care Unit (PACU), Cardiac Care Unit (CCU), Paediatric Intensive Care Unit (PICU) shall be cared for by their respective medical officer and specialist in charge of each CIC unit.
- ii. The clinical management of patients in the Intensive Care unit shall be guided by management protocols in Intensive Care and other relevant guidelines/protocol.
- iii. Nursing personnel stationed in CIC and nursing care shall be supervised and managed by their respective CIC Nurse Officer/Nurse Supervisor/Nurse Manager.
- iv. All categories of staff shall be credentialed and privileged to perform specific tasks appropriate to their level of skills and competency.
- v. Communication with the patient and family members is essential and there shall be full disclosure of any unexpected adverse outcome by the attending Specialist.
- vi. Communication among the Healthcare Providers shall be enhanced. Intra and

inter-departmental communication shall be open, honest and effective to ensure optimal patient care. Staff shall display respect and tolerance towards others to maintain a harmonious interpersonal relationship.

5.6 Forensic Services

- i. Forensic and mortuary services shall be provided by the Hospital. The service shall be under the responsibility of a Forensic Pathologist appointed as the Head of Department by the Hospital Director. He/she will be involved in the planning, management, and provision of quality and safe forensic services.
- ii. The services shall include but not limited to the provision of body reception, body storage, body preparation area, area for viewing and bereavement and autopsy suites.
- iii. Complete records shall be maintained and include the following:
 - Registration of bodies received.
 - Registration of police and non-police cases.
 - Records of the body released to the next of kin including transport to the final destination.
 - Release and disposal records of unclaimed bodies.
 - Records of forensic and clinical post mortem cases.
 - Records of post mortem report requests and court proceedings.
- iv. In forensic post mortem, the chain of custody of the deceased person, trace evidence and specimens obtained during the procedure shall be maintained at all times, as required by the law.
- v. All body parts and foetus in the Hospital shall be registered at the mortuary. Body parts and foetus shall be released to the claimant through the mortuary. All information on body parts and foetus release shall be documented.

5.7 Medical Laboratory Services (MLS)

- i. Laboratory services are performed in the Medical Laboratory. The services shall be organised and administered to provide high-quality diagnostic services for safe patient care.
- ii. Department of Pathology and Medical Laboratory shall provide an up-to-date Medical Laboratory Handbook for staff and customer references as a guide on the services offered, specimen types, specimen volume and container, specimen handling and transportation and Laboratory Turn Around Time (LTAT).
- iii. Tests shall only be requested by authorised personnel involved in patient management. The request is made using the Computerised Order Entry in the integrated Total Hospital Information System (THIS)/Laboratory Information System (LIS). Requests for transfusion medicine services are made using hard-copy forms.

- iv. Specimen collection shall follow the guideline provided in the laboratory handbook. A pneumatic tube system shall be used whenever possible. The department is responsible for monitoring the transportation of the samples to the laboratory to ensure the quality of test results is maintained.
- v. The system shall enable the validation of all relevant tests by identified competent persons.
- vi. Important steps in specimen movement/process shall be recorded in the THIS/LIS. The automatic flow of information to/from analyzer machines is feasible in an integrated THIS.
- vii. All results shall be viewed electronically via the Electronic Medical Record (EMR). The reports of the investigations requested by other hospitals shall be made by the requesting physicians and sent via a mutually agreed and safe format.
- viii. Standardisation of practices and procedures shall be implemented in all the laboratories where possible.
- ix. The testing process shall be monitored through the calibration process, internal and external quality control programs and periodic maintenance according to equipment requirements.
- x. Clinical interpretation of test result/report is made by clinically qualified personnel (Pathologist). The laboratory shall notify the ward/clinic of abnormal test results exceeding “critical values” that are established at the national level. There shall be a documented procedure for handling and reporting urgent requests.
- xi. The outsourcing of services shall be arranged with accredited or technically competent referral laboratories through the Department of Pathology and Medical Laboratory. The department shall maintain a list of referral laboratories.
- xii. Point of Care Testing (POCT) equipment is under the responsibility of the users and department, and shall follow the national guideline.^[52]
- xiii. Laboratory safety practices shall comply with the existing laboratory safety requirements and all relevant statutory acts and regulations. All medical laboratory personnel shall be given adequate training in laboratory safety.
- xiv. The Medical Laboratory shall be accredited by a renowned accreditation body to ensure delivery of services of the highest standard.

5.8 Medical Imaging Services

- i. Radiological services will be provided for referrals, walk-in and in and out-patients.
- ii. All radiology procedures shall be performed by qualified and credentialed personnel. Notwithstanding the above, medical personnel who have undergone appropriate training in specific procedures may be privileged to perform the procedures.
- iii. A radiological investigation or procedure shall be performed upon request from a registered medical/dental practitioner and when deemed appropriate by a Radiologist. Such a request shall contain clinical information to justify the examination.
- iv. All requests for radiology examinations shall be accompanied by duly completed

^[52] National Point of Care Testing Policy and Guidelines, 2012, MOH

radiology request forms/order entries (including consent and checklist, if relevant).

- v. The Department shall be fully operational for all types of examinations during office hours.
- vi. Outside office hours, radiological examinations shall be performed according to urgency.
- vii. Examinations on a patient shall be carried out in the presence of an appropriate chaperone (if applicable).
- viii. For all radiological examinations involving ionising radiation, the dose/exposure factors/fluoroscopy time shall be recorded.

Chapter 6:

Hospital Amenities

6.1 Staff Facilities

- i. Staff facilities shall either be allocated to individuals or commonly shared by all staff.
- ii. Doctors on-call shall be provided with on-call rooms.
- iii. Staff clinic shall be established to provide primary medical care for Hospital staff.

6.2 Public Facilities

- i. Public facilities shall be under the responsibility of Hospital Management. Facilities available for public use include:
 - Visitors' Lounge (*Balai Pelawat*)/Discharge Lounge
 - Prayer Rooms (*Surau*)
 - Breastfeeding Room/Baby Changing Room
 - Cafeteria
 - Toilets including for the disabled
 - Shops/Vending Machines
 - Auto-Teller Machines (ATM)
 - Police Booth
 - Parking
- ii. Visitors' Lounge (*Balai Pelawat*)/Discharge Lounge and other designated waiting areas for the patients' relatives shall be opened over 24 hours. Those who use the lounge shall be subjected to the rules and regulations of the Hospital.
- iii. Prayer rooms shall be opened over 24 hours to the public and staff.
- iv. A common cafeteria located in the area of the Hospital is available for staff and the public. The cafeteria shall be operated by food service contractors appointed by the Hospital.
- v. Shops are available at the main Hospital building.
- vi. Food delivery services are only allowed in designated areas.

6.3 Car Park

- i. All Hospital staff shall park their vehicles at the designated staff parking areas.
- ii. Only cars with HPUniSZA staff stickers shall be allowed to enter the staff parking area. All staff vehicles must display the HPUniSZA staff sticker.
- iii. Ambulances and other Hospital vehicles shall be parked at designated areas.
- iv. Designated disabled person's parking areas shall only be used by a disabled person identified by an OKU sticker or card. Unauthorised vehicle parking in this area shall be clamped by security personnel.

- v. No parking shall be allowed at the entrance of ETD.
- vi. Designated police, fire & rescue vehicles parking shall be made available within the vicinity of ETD.
- vii. The Hospital or UniSZA shall not be responsible for the safety of the vehicles. Signage shall be displayed to the public that vehicles are parked at their own risk.
- viii. Access to the Hospital complex from car parks shall be through the main entrance, drop-off entrances and staff-designated entrances.
- ix. The traffic shall be directed to the respective entrances of the Hospital namely ETD, Specialist Clinics, Main Lobby, Obstetrics and Gynaecology Assessment Centre (OGAC), Rehabilitation Medicine, Psychiatry Department and Oncology.
- x. Management of car parks shall be carried out by the Department of Engineering and Biomedical and may appoint an outsourced company to run the car park services.

Chapter 7:

Quality Management

7.1 Standards

- i. The Hospital's performance in quality care shall adhere to the Malaysian Society for Quality in Health (MSQH) Hospital Accreditation Standards and Key Performance Indicators (KPI).
- ii. All cases of Shortfall in Quality (SIQ) shall be investigated to identify the cause and carry out remedial action.
- iii. The Hospital shall establish its specific indicators for monitoring quality within the Hospital departments/units.

7.2 Quality Improvement Activities

- i. The Hospital shall establish the Quality and Standards Unit that oversees the quality initiatives of the Hospital. This department shall be led by a Clinical Specialist appointed by the Hospital Director. Coordinators shall be appointed for the different activities.
- ii. The following quality activities shall be implemented:
 - Quality Assurance studies
 - Quality Control Circle
 - Malaysian Patient Safety Goals
 - Incident reporting
 - Hospital Accreditation certification
 - Clinical Audits
 - Mortality Reviews (suspicious deaths, perioperative and postoperative deaths, maternal mortality, perinatal and neonatal mortality, specific communicable diseases such as dengue, tuberculosis, leptospirosis and others in accordance with Notifiable Disease Act.^[46]
 - Root Cause Analysis (RCA)
 - Pain-free Hospital
 - Safe Surgery Saves Lives (SSSL)
- iii. Every Clinical and Non-Clinical Department is responsible for its Quality Programme to ensure continuous improvement in the delivery of health care services.

7.3 Training

7.3.1 Continuous Professional Development Programmes

- i. The Hospital management shall facilitate Continuous Professional Development (CPD) activities in the Hospital.
- ii. The Hospital shall establish a Training Unit and Training Committee to

^[46] Prevention and Control of Infectious Disease Act 1988, MOH.

provide the direction and governance for the CPD programme.

- iii. Each personnel (both administrative and clinical) will be allowed to attend training programmes in areas relevant to their functions, for not less than 7 days to maintain staff competency, which includes technical, soft skills and communication skills. Where applicable, logbooks or online CPD activities should be updated.
- iv. Staff shall be responsible for his/her professional development to improve work performance. The Head of Department/Unit shall suggest appropriate training for individual staff to develop his/her knowledge and skill.
- v. Sufficient funding and other resources which may include the library, auditorium, seminar room, skills lab, computer lab etc. will be established in the Hospital.
- vi. The Hospital shall establish formal and informal linkages and collaborations with local and international health-related organizations to facilitate training activities.
- vii. Databases of in-house and external training programmes organised and/or attended by each person must be maintained and updated by the Training Unit.

7.3.2 Credentialing & Privileging (C&P)

- i. The Hospital shall establish committees consisting of clinicians and senior allied health staff to coordinate credentialing & privileging (C&P) processes:
 - Medical and Surgical Advisory Committee (MSAC)
 - HPUiSZA C&P Committee
 - HPUiSZA C&P Technical Committee for Medical Practitioners
 - HPUiSZA C&P Technical Committee for Clinical Support Staff
- ii. The C&P committee shall also identify procedures requiring additional training and experience in particular;
 - Invasive procedures requiring skill and knowledge
 - High-risk procedures
 - New technologies/techniques
 - Complex procedures
 - Procedures where the appropriateness of indications for use are open to abuse.
- iii. Each speciality shall be responsible for determining the “core clinical activities” within the speciality.

- iv. Each speciality shall be responsible for the identification of training requirements for its subspecialty, the organisation of training courses and workshops.
- v. The C&P Committee shall determine and delineate the clinical privileges of a healthcare provider within the said department, consistent with his or her credentials, clinical competency and the Hospital's needs, capability and resources.
- vi. The C&P committee shall periodically review the privileges granted after a defined period.
- vii. The Head of Department/Unit is responsible for verifying and assessing professional performance, peer recommendation, clinical judgments and/or technical skills.
- viii. A practitioner whose credentialed status is denied, withdrawn or limited has the right to appeal to the Medical & Surgical Advisory Committee (MSAC) of the Hospital. The Hospital Director's decision, following consultation with members of the MDAC, will be final and binding.
- ix. All visiting clinicians shall apply for credentialing and privileging from the HPUiSZA C&P Committee. Refer to existing practice guidelines.^{[47][48]}

7.3.3 House Officers and Other Post-Basic/Graduate Training/Master Programme and Subspecialty Training

- i. As the Hospital also serves as the training centre for undergraduates/ House Officers and/or other post-basic/graduate programmes, the Hospital is required to establish a formalised training and assessment structure relevant to the type of training being provided.
- ii. Compliance with the Medical Staff bylaws shall also be observed.

7.4 Research

- i. The Hospital shall promote, support and facilitate the conduct of clinical and non-clinical research.
- ii. All research-related activities at the Hospital shall be registered and under the supervision of the Head of Research, Clinical Research Unit HPUiSZA.
- iii. All research must be registered at the Clinical Research Unit Office before the commencement of the study and the office shall facilitate the conduct of research.
- iv. All clinical research conducted at the Hospital must be registered and approved by UniSZA. Also, the study must be sanctioned by the Hospital Director and the respective Head of Department.
- v. All non-clinical research conducted at the Hospital must be registered and the study must be approved by the Hospital Director.

^[47] SPKPK Bil.11/2008 Panduan Penggunaan Khidmat Doktor Swasta Untuk Perkhidmatan di Klinik Kementerian Kesihatan (Hospital dan Klinik Kesihatan) dengan Kadar Baru RM80 sejam, 24 August 2008.

^[48] SPKPK Bil.4/2001 Garis Panduan Pengambilan Pakar Swasta untuk Berkhidmat di Hospital-Hospital Kerajaan, 22 February 2001.

- vi. All Principles Investigator (PI), Co-investigator and Research Collaborators, who undertake interventional clinical trials must possess a Malaysian Good Clinical Practice (MGCP) certificate.
- vii. All Interventional clinical trials must include a PI or Co-investigator from the Hospital.
- viii. Any presentation (oral/poster) or publication, e.g. research reports, abstracts, manuscripts, or conference proceedings by the Hospital staff must be registered with the Clinical Research Unit.
- ix. The Hospital shall maintain the medical records of patients involved in clinical trials.
- x. All other relevant research documents shall be maintained by units under the purview of the Clinical Research Unit for audit and monitoring. Management of such records by the PI and the sponsor shall be governed by Good Clinical Practice.

Chapter 8:

Disaster Management

8.1 Disaster & Emergency Preparedness

- i. There shall be a Hospital Disaster Committee headed by the Hospital Director.
- ii. The members of the committee shall include Deputy Director (Clinical Support), clinicians from various relevant departments and administrators.
- iii. The Committee shall develop a Hospital Disaster Management Plan which includes *External Disaster Action Plans, Internal Disaster Action Plans and Individual Department Disaster Action Plans*.
- iv. The Committee shall be involved in all the phases of Disaster Management (Prevention, Preparedness, Response and Recovery).

8.2 Phases of Disaster Management

8.2.1 Prevention Phase

- i. The Hospital Disaster Committee shall be responsible for the activities involved in the prevention phase of disaster management.
- ii. The plan shall include the following:
 - Risk identification of potential internal and external hazards of the Hospital;
 - Risk communication to the various stakeholders within the Hospital, State Health Office and Ministry of Health, UniSZA, and Ministry of Higher Education (MoHE).
 - Response capacity assessment.
- iii. The Hospital Disaster Committee shall propose structural and non-structural preventive measures that can be implemented by the Hospital to minimise the hazard threat.

8.2.2 Preparedness Phase

The Hospital shall have a comprehensive plan which includes a contingency plan and Standard Operating Procedure (SOP) in handling any emergency or disaster. Reference may be made to available directives and guidelines.^{[49][50][51]}

- i. The Hospital Disaster Management Plan shall be developed during this phase.
- ii. The Committee shall meet on a regular basis and shall be responsible for the development and implementation of the hospital contingency plan and SOP pertaining to disaster preparedness.
- iii. The Hospital Disaster Management Plan shall be communicated to all staffs.
- iv. All departments shall have their SOP and evacuation plan in the event of any emergency or disaster. An exit route plan is to be displayed at strategic locations in every Department/ Unit/ Ward including the assembly areas.
- v. Staff shall be trained on the use of equipment, patient transportation and

[49] *Pelan Pengurusan Bencana Peringkat Kementerian Kesihatan Malaysia (Refer to Sektor Pengurusan Wabak & Bencana KKM, Mac 2015).*

[50] *Pelan Tindakan Insiden Kecemasan dan Bencana Dalaman Bagi Hospital-Hospital KKM (Refer to Medical Development Division, Ministry of Health 2018).*

[51] *Arahan 20, Majlis Keselamatan Negara (Dasar dan Mekanisme Pengurusan Bencana Negara).*

evacuation processes etc.

- vi. Hospital mock disaster drills and table-top exercises shall be carried out annually and evaluated accordingly.
- vii. Business Continuity Plan (BCP) shall be included in the Hospital Disaster Management Plan.

8.2.3 Response Phase

- i. In the event of a disaster, the disaster declaration of the Hospital shall be made by the Hospital Disaster Committee and activated according to the Hospital Disaster Management Plan.
- ii. The Hospital Disaster Management Plan shall be carried out in a systematic and planned manner.
- iii. Disaster Operation Room shall be created in the ETD and an administrative area in the event of a disaster.
- iv. For an internal disaster such as fire, earthquake, bomb threat etc, the hospital evacuation plan shall be carried out.
- v. The BCP shall be implemented during this phase.

8.2.4 Recovery & Assessment Phase

- i. When the disaster has passed, the Hospital Director shall declare the stand-down order, according to the Hospital Disaster Management Plan.
- ii. The Recovery & Assessment Phase shall be carried out according to the SOP lined out in the Hospital Disaster Management Plan.
- iii. The committee shall prepare a post-mortem report of the incident or disaster.
- iv. The Committee shall identify and assist in the treatment of Post-Traumatic Stress Disorder (PTSD) among the staff.

8.3 Specific Contingency Plan

- i. *A specific contingency plan shall be available for the following situations based on the individual departments:
 - Power failure
 - IT system breakdown
 - Lift breakdown
 - Disruption in water supply
 - Gas leakage
 - Flood
 - Disease outbreak
 - Air condition failure
 - Building infestation
 - Earthquake

- Tele-Communication Failure (Private Automatic Branch Exchange (PABX) shutdown)
- ii. The plan includes notifications, allocation of responsibilities, immediate actions, alternative solutions and follow-up measures.
- iii. Staff will be briefed on the plan and appropriate training shall be carried out.

*The contents contained herein is subject to change at any time.



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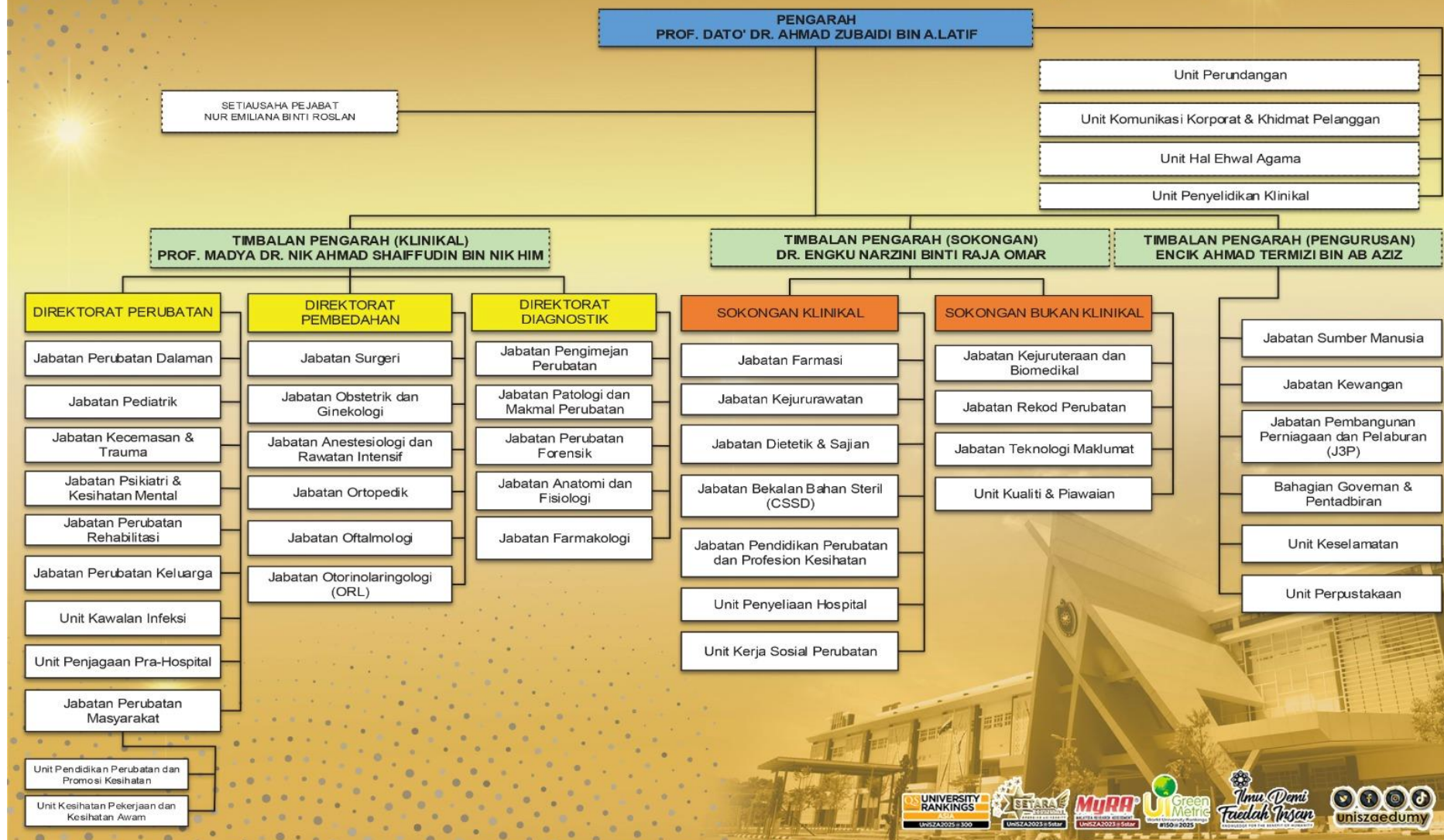
APPENDICES 1 & 2

- 1: Organisational Chart
- 2: List of Committees



CARTA ORGANISASI

HOSPITAL PENGAJAR UNIVERSITI SULTAN ZAINAL ABIDIN (HPUniSA)



SENARAI JAWATANKUASA DI BAWAH HOSPITAL PENGAJAR UNIVERSITI

PERINGKAT	JAWATANKUASA			
TIER- 1 (STRATEGIK / DASAR)	LEMBAGA PENGARAH UNIVERSITI (LPU) Meluluskan dasar dan memantau prestasi dan terlibat dengan membuat keputusan penting.			
TIER- 2 (STRATEGIK / DASAR)	JAWATANKUASA PENGURUSAN HOSPITAL PENGAJAR UNIVERSITI (JPHU) Bertanggungjawab dalam pemantauan pelaksanaan keputusan Lembaga Pengarah Universiti (LPU) bagi hal ehwal Hospital Pengajar Universiti.			
TIER-3 (EKSEKUTIF)	JAWATANKUASA PENGURUSAN UNIVERSITI (JKPU) Bertanggungjawab mengawal dan menyelia operasi Hospital Pengajar Universiti.			
TIER-4 (OPERASI)	MESYUARAT PENGURUSAN HOSPITAL PENGAJAR UNIVERSITI (MPHU) Bertanggungjawab melaksanakan dasar dan hal ehwal operasi peringkat Hospital Pengajar Universiti.			
OPERASI	JAWATANKUASA-JAWATANKUASA DI BAWAH HOSPITAL PENGAJAR UNIVERSITI			
	Pengurusan	Pentadbiran & Sumber Manusia	Pengurusan Kewangan	Pengurusan Klinikal dan Sokongan Klinikal

	*Jawatankuasa-Jawatankuasa Di Bawah Hospital Pengajar Universiti			
OPERASI	Pengurusan	Pengurusan Sumber Manusia & Pentadbiran	Pengurusan Kewangan	Pengurusan Klinikal dan Sokongan Klinikal
	1. JK Patuh Syariah (Urusetia: Unit Hal Ehwal Agama) 2. JK Aduan dan Kepuasan Pelanggan (Urusetia: Unit Komunikasi Korporat & Khidmat Pelanggan) 3. JK Penjanaan	1. Panel Pembangunan Sumber Manusia (PPSM) (Urusetia : Bahagian Pembangunan Sumber Manusia, Jabatan Sumber Manusia & Pentadbiran) 2. JK Latihan (Urusetia: Unit Latihan) 3. JK Cuti Belajar (Urusetia : Unit Kemajuan Kerjaya) 4. JK Tabung Amanah (Urusetia: 5. JK Tabung Kebajikan Staf (Urusetia : Unit Saraan, Kemudahan & Kebajikan) 6. Majlis Bersama Jabatan (menggunapakai JK yang sedia ada di universiti)	1. JK Tetap Kewangan dan Pelaburan (JTKP) 2. JK Pengurusan Kewangan dan Akaun (JKPA) 3. JK Audit dan Risiko (JKAR) 4. Lembaga Perolehan 5. JK Sebut Harga 6. JK Pembangunan 7. JK Bajet 8. JK Pengurusan Aset Alih Kerajaan (JKPAK) (JK yang dinyatakan dalam perkara 1-8 adalah menggunapakai JK yang telah tersedia di Universiti) 9. JK Caj Pesakit 10. JK Pengurangan dan Pengecualian Caj Hospital	1. JK Lembaga Perubatan 2. JK Penasihat Perubatan (MAC) 3. JK Tabung Kebajikan Pesakit (Urusetia: Unit Kerja Sosial Perubatan) 4. JK Dewan Bedah 5. JK Resusitasi (Urusetia: Jabatan Kecemasan & Trauma) 6. JK Perlindungan Sinaran (Urusetia: Jabatan Pengimejan Perubatan) 7. JK Ruang & Keceriaan Hospital 8. JK Ubat dan Terapeutik. (Urusetia: Jabatan Farmasi)

				9. JK Point of Care Testing (Urusetia: Jabatan Patologi & Makmal Perubatan) 10. JK Penyelidikan Klinikal (Urusetia: Unit Penyelidikan Klinikal) 11. JK Kawalan Infeksi (Urusetia: Unit Kawalan Jangkitan) 12. JK Mortaliti dan Morbiditi (Urusetia: Jabatan Anestesiologi & Rawatan Intensif) 13. JK Transfusi (Urusetia: Jabatan Patologi & Makmal Perubatan) 14. JK Pengurusan Bencana (Urusetia: Jabatan Kecemasan & Trauma) 15. Jawatankuasa <i>Credentialing & Privileging (C&P)</i> 16. Jawatankuasa Rekod Perubatan (MRC) 17. Jawatankuasa Kawalan Radiasi
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				18. Jawatankuasa Direktorat Surgikal 19. Jawatankuasa <i>Continuous Professional Development (CPD)</i> 20. Jawatankuasa Kualiti Makanan 21. Jawatankuasa Pendermaan Organ 22. Jawatankuasa Validasi 23. JK <i>Annual Practising Certificate</i> 24. JK Keselamatan & Kesihatan Pekerja 25. JK Etika dan Medicolegal 26. JK Pemantau Perkhidmatan Sokongan
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*These current committees established for the hospital development. Later on, there will be more committees to be established to ensure the smoothness of hospital's operation.

4

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5

LIST OF ABBREVIATIONS

ABBREVIATIONS	DEFINITION
AIDS	Acquired Immunodeficiency Syndrome
AMO	Assistant Medical Officer
AMR	Antimicrobial Resistance
ARC	Admission and Revenue Counter
ATM	Auto-Teller Machine
BBA	Birth Before Arrival
BCP	Business Continuity Plan
BIA	Birth in Ambulance
BID	Brought in Dead
BMS	Balance Medication Sheet
BYOD	Bring Your Own Device
C&P	Credentialing and Privileging
CA	Concession Agreement
CCaCS	Corporate Communication and Customer Service
CCORM	UniSZA Corporate Communication Centre
CCU	Cardiac Care Unit
CIC	Centralised Intensive Care
CME	Continuous Medical Education
CPD	Continuous Professional Development
CSI	Crime Scene Investigation
CSSD	Central Sterile Service Department

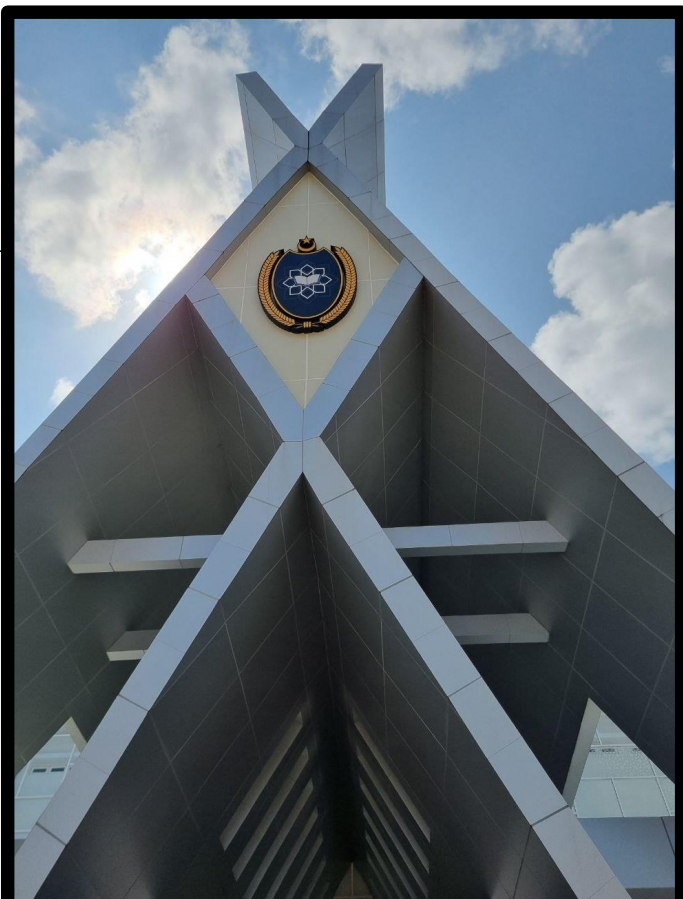
ABBREVIATIONS	DEFINITION
DAMA	Discharge Against Medical Advice
DDA	Dangerous Drug Act
DG	Director of Health
DIL	Death in Line
DNR	Do Not Resuscitate
DS	Discharge Summary
E-GL	Letter of Guarantee
EMR	Electronic Medical Record
ES	Encounter Summary
ETD	Emergency and Trauma Department
GCP	Good Clinical Practice
GMP	Good Manufacturing Practice
GP	General Practitioner
HACCP	Hazard Analysis and Critical Control Point
HBOT	Hyperbaric Oxygen Therapy (HBOT)
HBV	Hepatitis B
HCAI	Healthcare Associated Infection
HCV	Hepatitis C
HCWs	Healthcare Workers
HDU	High Dependency Unit
HIACC	Hospital Infection and Antibiotic Control Committee

ABBREVIATIONS	DEFINITION
HIV	Human Immunodeficiency Virus
HO	House Officer
HOD/HOU	Head of Department/Head of Unit
HPUniSZA	Hospital Pengajar Universiti Sultan Zainal Abidin
HSIP	Hospital Specific Implementation Plan
ICD	International Classification of Disease
ICT	Information & Communication Technology
ICU	Intensive Care Unit
JBU	Jawatankuasa Bajet Universiti
JCP	Jawatankuasa Caj Pesakit
JKKP	Jawatankuasa Keselamatan dan Kesihatan Pekerja
OSHA	Department of Occupational Health and Safety
JKPAK	Jawatankuasa Pengurusan Aset Kerajaan
JKPP	Jawatankuasa Pengurangan dan Pengecualian Caj
JKPU	Jawatankuasa Pengurusan Universiti
JPHU	Jawatankuasa Pengurusan Hospital Universiti
JPKA	Jawatankuasa Pengurusan Kewangan dan Akaun
JTM	Department of Information Technology
KPI	Key Performance Indicators
LIS	Laboratory Information System
LPU	Lembaga Pengarah Universiti

ABBREVIATIONS	DEFINITION
LRPG	Laws, Regulation and Practice Guidelines
LTAT	Laboratory Turn Around Time
MAMPU	Malaysian Administrative Modernisation and Management Planning Unit
MAP	Master Agreed Procedures
MC	Medical Certificate
MDROs	Multidrug-Resistant Organisms
MGCP	Malaysian Good Clinical Practice
MHAM	Medical and Humanitarian Aid Mission
MLS	Medical Laboratory Services
MMC	Malaysian Medical Council
MO	Medical Officer
MOA	Memorandum of Association
MOH	Ministry of Health
MoHE	Ministry of Higher Education
MOU	Memorandum of Understanding
MPHU	Mesyuarat Pengurusan Hospital Universiti
MRC	Medical Record Committee
MRO	Medical Record Officer
MSAC	Medical and Surgical Advisory Committee
MSQH	Malaysian Society for Quality in Health
MSW	Medical Social Work

ABBREVIATIONS	DEFINITION
MTC	Malaysian Triage Category
NDA	Non-Disclosure Agreement
NGO	Non-Governmental Organisation
NICU	Neonatal Intensive Care Unit
O&G	Obstetrics and Gynaecology
OGAC	Obstetrics and Gynaecology Assessment Centre
OSHA	Occupational Safety and Health
OT	Operation Theatre
PA	Public Address
PABX	Private Automatic Branch Exchange
PACS	Picture Archiving Communication System
PACU	Post-anaesthesia Care Unit
PDPA	Personal Data Protection Act
PEP	Post Exposure Prophylaxis
PI	Principal Investigator
PICU	Pediatric Intensive Care Unit
PKU	Pusat Kesihatan Universiti
POCT	Point of Care Testing
PPE	Personal Protective Equipment
PPM	Planned Preventive Maintenance
PRO	Public Relations Officer

ABBREVIATIONS	DEFINITION
PTSD	Post-Traumatic Stress Disorder
RCA	Root Cause Analysis
RIS	Radiology Information System
RN	Running Number
RPC	Radiation Protection Committee
RPO	Radiation Protection Officer
SCN	Special Care Nursery
SIQ	Shortfall in Quality
SMS	Short Message Services
SOCSSO	Social Security Organisation
SOP	Standard Operating Procedure
SSSL	Safe Surgery Saves Lives
THIS	Total Hospital Information System
TRPI	Technical Requirement and Performance Indicator
UACP	User Access Control Policy
UMC	University Medical Centre
UMT	Universiti Malaysia Terengganu
UniSZA	Universiti Sultan Zainal Abidin
VIP	Very Important Person



Hospital Pengajar Universiti Sultan Zainal Abidin General Hospital Operational Policy is comprehensive and authoritative: It presents the essential information and uniformity required for best level of care in the hospital. It also serves as a guide for doctors, allied health professionals and hospital administrators in understanding their roles and responsibilities within the organization. This policy book will prove invaluable to all hospital staff in ensuring standardized practices in day-to-day running of the hospital, paving the way for Quality and Standards Unit HPUiSZA.

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